

BASELINE SURVEY REPORT

Building a Resilient and Inclusive Regional Social Movement to Oversee the Implementation of SRHR Commitments

KENYA • UGANDA • TANZANIA

Initial baseline on the structures, formations and capacities of sexual and reproductive health and rights social movements



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TABLE OF CONTENTS

CHAPTER ONE: INTRODUCTION	7
1.1. ABOUT EACHRIGHTS.....	8
1.2. PROJECT BACKGROUND AND CONTEXT.....	8
1.3. IMPLEMENTING PARTNERS	9
1.4. OBJECTIVES OF THE BASELINE SURVEY	9
1.5. TECHNICAL APPROACH	10
1.6. SCOPE OF THE BASELINE SURVEY.....	11
CHAPTER TWO: BASELINE METHODOLOGY.....	12
2.1. EVALUATION DESIGN AND APPROACH	12
2.2. PLANNING AND INCEPTION	12
2.3. LITERATURE REVIEW AND POLICY ANALYSIS	13
2.4. SAMPLING TECHNIQUES.....	13
2.5. DATA COLLECTION TECHNIQUE AND APPROACH.....	14
A) KEY INFORMANTS WITH GEM TRUST	14
B) KEY INFORMANTS WITH CEHURD	14
C) KEY INFORMANTS WITH CWHRDS.....	14
2.6. DATA MANAGEMENT AND QUALITY ASSURANCE.....	14
2.6.1. ANALYSIS AND INTERPRETATION.....	14
2.6.2. DATA QUALITY ASSURANCE.....	15
2.7. ETHICAL AND SAFEGUARDING PRINCIPLES	15
2.7.1. ETHICAL CONSIDERATIONS	15
2.7.2. SOCIAL INCLUSION AND INTEGRATING GENDER CONSIDERATIONS	16
2.8. LIMITATIONS AND MITIGATIONS	16
CHAPTER THREE: BASELINE FINDINGS.....	17
3.1. INTRODUCTION	17
3.2. SRHR LEGAL AND POLICY LANDSCAPE	17
3.2.1 REGIONAL ANALYSIS	17
3.2.2 KENYA.....	18

3.1.2. UGANDA.....	21
3.1.3. TANZANIA.....	23
3.2 BASELINE INDICATORS ANALYSIS.....	25
INDICATOR 1: AMPLIFYCHANGE GRANTEE UNDERTAKES RIGOROUS ADVOCACY PLANNING AND/OR REGULAR ADVOCACY PLAN REVIEWS FOR SRHR POLICY CHANGE.	25
INDICATOR 2: INCREASED CSO PARTICIPATION IN POLICY AND BUDGETARY PROCESSES .	31
INDICATOR 3: AMPLIFYCHANGE GRANTEES STRENGTHEN THEIR OWN ORGANIZATIONAL CAPACITY	36
INDICATOR 4: PARTNERSHIPS ESTABLISHED WITH NON-TRADITIONAL SRHR ALLIES	38
INDICATOR 5: INCLUSION OF CSO RECOMMENDATIONS IN POLICY AND BUDGETARY DECISIONS/PROPOSALS/REVIEWS.....	41
3.3 EXISTING STRUCTURES AND FORMATION OF SRHR SOCIAL MOVEMENTS	45
3.3.1 STRUCTURE OF SRHR SOCIAL MOVEMENTS IN KENYA, UGANDA AND TANZANIA	45
3.3.2 FORMATION OF SOCIAL MOVEMENTS IN KENYA, UGANDA AND TANZANIA.....	46
3.3.2. TYPES OF SRHR SOCIAL MOVEMENTS.....	47
3.3.3. SRHR COALITIONS AND THEMATIC FOCUS	48
3.3.5 GOVERNANCE AND LEADERSHIP MODELS	50
3.4 ORGANIZATION AND ADVOCACY CAPACITIES	52
3.4 COORDINATION PLATFORMS AND MECHANISMS	52
3.4.1. REGIONAL COORDINATION	52
3.4.2. COUNTRY LEVEL COORDINATION.....	53
3.5 CROSS CUTTING CHALLENGES AND OPPORTUNITIES	56
3.5.5 CHALLENGES	56
3.5.6 ENTRY POINTS AND OPPORTUNITIES	57
CHAPTER FOUR: CONCLUSIONS & RECOMMENDATIONS	60
4.1. CONCLUSIONS.....	60
4.2. RECOMMENDATIONS	61
ANNEXES	64
ANNEX I: BASELINE INDICATOR MATRIX.....	64
REFERENCES	77

ACRONYMS

AFHS	Adolescent-Friendly Health Services
AFRA	Accountability for Reproductive Health Advocacy (Kenya)
AFYA	Swahili for "Health" (used in SRHR programme branding e.g., Afya Plus)
AGYW	Adolescent Girls and Young Women
AIDS	Acquired Immune Deficiency Syndrome
AMREF	African Medical and Research Foundation
APDK	Association for the Physically Disabled of Kenya
ASAL	Arid and Semi-Arid Lands
ASRH	Adolescent Sexual and Reproductive Health
AU	African Union
AWCFS	African Woman and Child Feature Service
BDPfA	Beijing Declaration and Platform for Action (1995)
CDC	Centres for Disease Control and Prevention
CEDAW	Convention on the Elimination of All Forms of Discrimination Against Women
CEHURD	Center for Health, Human Rights and Development (Uganda)
CHESA	Community Health Education Services and Advocacy (Tanzania)
CIVICUS	World Alliance for Citizen Participation
CSE	Comprehensive Sexuality Education
CSMMUA	Coalition to Stop Maternal Mortality Due to Unsafe Abortion (Uganda)
CSO	Civil Society Organisation
COMESA	Common Market for Eastern and Southern Africa
CREAW	Centre for Rights Education and Awareness
CWHRD CWHRDs	/ Coalition for Women Human Rights Defenders
DASH	Division of Adolescent and School Health
DAYO	Dream Achievers Youth Organization
DHMTS	Department of Health Management and Technical Services
EAC	East African Community
EACHRights	East African Centre for Human Rights
EALA	East African Legislative Assembly
EANNASO	Eastern Africa National Networks of AIDS Service Organisations
EAWHRDN	East Africa Women Human Rights Defenders Network

ECOSOC	United Nations Economic and Social Council
ESA	Eastern and Southern Africa
ESARO	Eastern and Southern Africa Regional Office (UNFPA/UNICEF)
FAWE	Forum for African Women Educationalists
FemAct	Female Activist Coalition
FEMNET	African Women's Development and Communication Network
FGD	Focus Group Discussion
FGM	Female Genital Mutilation
FIDA-Kenya	Federation of Women Lawyers – Kenya
FLEN	Family Life Education Network
GALCK+	Gay and Lesbian Coalition of Kenya
GBV	Gender-Based Violence
GEM Trust	African Gender and Media Initiative Trust
GRREAT	Global Reach for Reproductive Empowerment and Advocacy Tool
HIV	Human Immunodeficiency Virus
HRAPF	Human Rights Awareness and Promotion Forum (Uganda)
ICPD	International Conference on Population and Development
ICPD25	25th Anniversary of the International Conference on Population and Development
IGAD	Intergovernmental Agency for Development
IOM	International Organisation for Migration
IPPF	International Planned Parenthood Federation
KELIN	Kenya Legal & Ethical Issues Network
KII	Key Informant Interview
KHRC	Kenya Human Rights Commission
KNCHR	Kenya National Commission on Human Rights
LGBTQIA+	Lesbian, Gay, Bisexual, Transgender, Queer/Questioning, Intersex, Asexual +
MEL	Monitoring, Evaluation and Learning
MoH	Ministry of Health
MoU	Memorandum of Understanding
MPoA	Maputo Plan of Action
NASCOP	National AIDS and STI Control Programme
NAYA Kenya	Network for Adolescent and Youth of Africa – Kenya
NGEC	National Gender and Equality Commission
NGO	Non-Governmental Organisation

NGLHRC	National Gay and Lesbian Human Rights Commission
NHIF	National Health Insurance Fund
NPA- VAWC	National Plan to End Violence Against Women and Children
NUWODU	National Union of Women with Disabilities of Uganda
NUDIPU	National Union of Disabled Persons of Uganda
P2P	Peer-to-Peer Uganda
PBO	Public Benefits Organisation
PLWHIV	Persons Living with HIV
PwDs	Persons with Disabilities
RAHU	Reach a Hand Uganda
RAP4SRHR	Rights Advocacy Project for Sexual and Reproductive Health and Rights
RHNK	Reproductive Health Network Kenya
RHRN	Right Here Right Now
RHU	Reproductive Health Uganda
RMHSU	Reproductive and Maternal Health Services Unit
RMNCAH	Reproductive, Maternal, Newborn, Child and Adolescent Health
SADC	Southern African Development Community
SDGs	Sustainable Development Goals
SGBV	Sexual and Gender Based Violence
SHA	Social Health Authority
SHIVYAWATA	Shirikisho la Vyama vya Watu Wenye Ulemavu Tanzania (Federation of Disabled Persons)
SMUG	Sexual Minorities Uganda
SRHR	Sexual and Reproductive Health and Rights
STI	Sexually Transmitted Infection
TGNP	Tanzania Gender Networking Programme
THRDC	Tanzania Human Rights Defenders Coalition
TWG	Technical Working Group
UHC	Universal Health Coverage
UDPK	United Disabled Persons of Kenya
UMATI	Family Planning Association of Tanzania (originally <i>Uzazi na Malezi Bora Tanzania</i>) — IPPF Member Association
UN	United Nations
UNAIDS	Joint United Nations Programme on HIV/AIDS
UNDP	United Nations Development Programme
UNESCO	United Nations Educational, Scientific and Cultural Organization

UNFPA	United Nations Population Fund
UNHCR	United Nations High Commission for Refugees
UNICEF	United Nations Children's Fund
UPR	Universal Periodic Review
UWONET	Uganda Women's Network
UYAHF	Uganda Youth and Adolescents Health Forum
WHRD	Women Human Rights Defender
WHO	World Health Organization
WLHIV	Women Living with HIV
WPI	Women's Probono Initiative (Uganda)
YFS	Youth Friendly Services

1.1. About EACHRights

The East African Centre for Human Rights (EACHRights) was founded in May 2010 and registered as a Trust in Kenya in November 2010. While the organisation acknowledges that all human rights are indivisible, interdependent, and interrelated, EACHRights implements programmes that promote, protect, and enhance human rights, with a special emphasis on realizing Economic and Social Rights, human dignity, and equitable development.

EACHRights operates as a non-partisan, regional non-governmental organisation, with a geographic focus on **Kenya, Uganda, and Tanzania**. Its mission is to promote social justice in East Africa through a combination of research, capacity building, advocacy, and public interest litigation.¹ EACHRights focuses its work on key thematic areas, including education, health, and sexual and reproductive health and rights (SRHR), climate justice and environment, economic justice and social protection, and business and human rights.

1.2. Project Background and Context

SRHR social movements in Kenya, Uganda, and Tanzania are shaped by distinct political, legal, and cultural contexts, yet share common goals of advancing bodily autonomy, equitable access to services, and rights-based advocacy. The desk review revealed a landscape of diverse actors and formations, including youth-led organizations, feminist networks, legal advocacy groups, and community-based initiatives.²

In Kenya, advocacy efforts span issues such as safe abortion, menstrual health, comprehensive sexuality education, and Lesbian, Gay, Bisexual, Transgender, Queer, Intersex and other diverse identities rights, led by a mix of grassroots and national-level organisations. Despite the vibrancy of the sector, coordination remains fragmented, with limited cross-sectoral collaboration and few sustained platforms for joint action.³

In Uganda, SRHR movements are anchored in strong legal and policy engagement, particularly around maternal health, adolescent SRHR, and access to services for key populations. However, restrictive laws and conservative social norms continue to constrain civic space and limit the reach of grassroots actors.⁴

In Tanzania, advocacy is challenged by a tightly regulated civic environment, where restrictions on freedom of expression and association pose significant barriers. Nonetheless, women's rights groups and youth collectives remain active in addressing gender-based violence, early marriage, and reproductive health access, often through informal networks and community-level mobilisation.⁵

¹ [EACHRights Official Website](#)

² [Amplify Change - Our Work](#)

³ [UPR Kenya SRHR Thematic Group Report \(2024\) - CSA Kenya](#)

⁴ [SRHR Alliance Uganda - Youth Demand Action](#)

⁵ [Restless Development – Policy Analysis of Adolescent SRHR in Tanzania \(2025\)](#)

In response to these realities, a three-year regional initiative titled “Building a Resilient and Inclusive Regional Social Movement to Oversee the Implementation of SRHR in Kenya, Uganda, and Tanzania” was launched with support from Amplify Change. The project is implemented in partnership with The African Gender and Media Initiative (GEM) Trust in Kenya, The Center for Health, Human Rights and Development (CEHURD) in Uganda, and The Coalition for Women Human Rights Defenders (CWHRDs) in Tanzania. It seeks to strengthen the capacity of SRHR social movements in the three countries to undertake coordinated advocacy through mechanisms such as the Universal Periodic Review (UPR).

1.3. Implementing Partners

African Gender and Media Initiative Trust (Kenya): GEM TRUST is a women-led research organization that works to advance gender equality through research and action on women’s human rights. GEM Trust began as a research arm and a reservoir of knowledge and information for the African Women and Child Feature Service (AWCFS) in media content development but has since evolved into a fully registered organization. GEM Trust brings together women experts in women’s human rights issues, specifically women’s health, sexual and reproductive rights, economic empowerment, media advocacy, and research. The advocates came together in response to the need for evidence-based programming in women’s human rights work in Kenya and Africa.

Center for Health, Human Rights and Development (Uganda): The Center for Health, Human Rights and Development (CEHURD) is a non-profit research and advocacy organisation that advances the right to health through law, policy engagement, evidence-based advocacy and community mobilisation. Its work includes strategic litigation, movement coordination and support to health-rights actors in Uganda and the wider East African region.

Coalition for Women Human Rights Defenders (Tanzania): The Coalition was founded in 2018 by a group of young women human rights activists and feminists who have witnessed the plight of violence and threats of women's human rights defenders who have been championing rights which are not yet recognized by legislations in the country; but which are identified, promoted and protected by international human rights instruments. The Coalition of WHRDs in Tanzania forms a large basis for advocacy and lobbying work conducted by WHRDs across the country. The types of activities undertaken by member organizations range from capacity building to service delivery.

1.4. Objectives of the Baseline Survey

The overall objective of the consultancy was to establish a comprehensive baseline. The specific objectives included the following;

- i. Establish the existing structures and formations of SRHR social movements in Kenya, Uganda, and Tanzania.

- ii. Assess the organizational and advocacy capacities of key actors engaged in SRHR advocacy across the three countries.
- iii. Map existing coordination platforms and mechanisms that facilitate collaboration and collective action at national and regional levels.
- iv. Identify gaps, opportunities, and entry points for strengthening regional SRHR advocacy efforts and enhancing the impact of social movements.

1.5. Technical Approach

The baseline survey applied a **multi-layered technical approach** to generate a comprehensive understanding of SRHR social movements in Kenya, Uganda, and Tanzania. The methodology was structured around four core objectives, each supported by tailored data collection strategies and analytical tools.

1. To **establish the existing structures and formations of SRHR social movements**, a stakeholder mapping exercise that identified both formal and informal movements, coalitions, networks, and collectives was conducted. This included documentation of governance models, legal status, thematic focus, and geographic reach, as well as an analysis of movement typologies such as youth-led, feminist, disability-inclusive, and LGBTIQ+ formations. The mapping built on due diligence insights and extended beyond core partners to capture the broader ecosystem.
2. To **assess the organizational and advocacy capacities of key actors**, a tailored capacity assessment tool was deployed and key informant interviews (KIIs) conducted with selected movement actors. The assessment examined technical, strategic, and operational capacities, advocacy experience, policy engagement, and coalition-building strength. It also explored internal systems such as monitoring and evaluation (MEL), resource mobilization, and communications, with attention to intersectionality and responsiveness to emerging SRHR issues.
3. To **map existing coordination platforms and mechanisms**, the survey utilized a coordination mapping tool and validated findings through interviews with platform hosts, regional conveners, and EACHRights' country partners. The analysis focused on the structure, frequency, inclusivity, and sustainability of coordination efforts, including cross-border linkages and digital advocacy channels. Informal coordination spaces and digital mechanisms were also explored to reflect the full spectrum of collaborative practices.
4. To **identify gaps, opportunities, and entry points for strengthening regional SRHR advocacy**, the survey synthesized findings from the first three objectives and applied a **SWOT analysis**. This process surfaced strategic blind spots, underrepresented issues and regions, and opportunities for synergy and capacity support. Validation sessions were convened with EACHRights, GEM Trust, CEHURD, and CWHRDs to co-create actionable recommendations and strategic levers for movement strengthening across the region.

1.6. Scope of the Baseline Survey

EACHRights conducted an initial project baseline survey aimed at establishing the status of SRHR social movements in Kenya, Uganda, and Tanzania. The survey conducted desktop research to examine existing literature and documentation on SRHR movements in the region, and assessed gaps, opportunities, and priority areas for advocacy. The survey also involved mapping the structures and relationships within and between SRHR movements, including their organizational configurations, advocacy strategies, resource mobilisation capacities, and collaborative networks.

Through this process, the consultant generated baseline indicators to support the measurement of project progress and effectiveness over time. These indicators were designed to inform strategic planning, track advocacy outcomes, and provide evidence-based recommendations for strengthening SRHR social movements. The baseline survey captured critical information to guide both the implementation and ongoing monitoring of the project.

This baseline report presents the scope, objectives, and methodological approach of the “Building a Resilient and Inclusive Regional Social Movement to Oversee the Implementation of SRHR in Kenya, Uganda, and Tanzania” project. It outlines the purpose and design of the baseline survey, the data collection and analysis methods employed, and provides an overview of how fieldwork was conducted. The report sets the foundation for understanding the status of SRHR advocacy ecosystems across the three countries at the project’s outset.

CHAPTER TWO: BASELINE METHODOLOGY

2.1. Evaluation Design and Approach

The baseline survey adopted a **qualitative evaluation design** to generate a context-specific and evidence-informed understanding of SRHR social movements in Kenya, Uganda, and Tanzania. The approach was guided by a structured methodology that prioritized depth, inclusivity, and contextual relevance, in line with the Terms of Reference and the framework outlined in the inception report.

A comprehensive **desk review** formed the foundation of the evaluation. The review identified formal and informal SRHR movements, coalitions, networks, and collectives; documented governance models, legal status, thematic focus, and geographic reach; and analyzed movement typologies such as youth-led, feminist, disability-inclusive, and LGBTIQ+ formations. It also informed the mapping of coordination platforms, digital advocacy spaces, and cross-border linkages, and helped surface strategic gaps and opportunities for regional alignment.

Primary data collection combined semi-structured key informant interviews (KIIs) with supplementary group-based evidence and validation discussions. Respondents included staff of the three implementing-partner civil society organisations, other civil society and movement actors, public-sector stakeholders, media practitioners, health and legal professionals, traditional leaders and other non-traditional allies. The late-November Uganda engagement and the Kenya capacity-building meeting in Machakos were incorporated as supplementary evidence sources and triangulated with the interview datasets and desk review.

Triangulation was undertaken across the three country KII workbooks, partner and contextual documents, the two attached Kenya group-work records, late Uganda validation evidence, the approved application, the inception report and the monitoring and evaluation plan. Supplementary group evidence was coded against the same themes as the KIIs. Where an attendance or consent register was not available in the attached evidence, the session informed thematic interpretation but was not added to the verified participant total.

2.2. Planning and Inception

The evaluation process commenced with a kick-off meeting convened by EACHRights, bringing together staff, consultants, and key implementing partners—GEM Trust (Kenya), CEHURD (Uganda), and CWHRDs (Tanzania). This collaborative session served to define the scope and parameters of the baseline survey, including agreement on the evaluation design, methodology, geographical coverage, and implementation timelines. The meeting also provided a strategic platform to clarify client expectations and outline the resources, documentation, and coordination mechanisms required to support successful delivery.

2.3. Literature Review and Policy Analysis

The desk review served as a foundational step in the baseline process, enabling the evaluation to contextualize field findings and identify thematic entry points for qualitative inquiry. This review involved a systematic examination of strategic reports, due diligence findings, organizational capacity assessments, and policy frameworks related to SRHR social movements in Kenya, Uganda, and Tanzania. It focused on identifying formal and informal SRHR movements, coalitions, networks, and collectives; documenting governance models, legal status, thematic focus, and geographic reach; and analyzing movement typologies such as youth-led, feminist, disability-inclusive, and other marginalized formations. Key resources included submissions to the Universal Periodic Review (UPR) process in Kenya, Uganda and Tanzania such as the SRHR Thematic Group Report and stakeholder submissions, which provided insight into national advocacy priorities and coordination gaps.

The review drew first on documentation from the three implementing partners: the African Gender and Media Initiative Trust (GEM Trust) in Kenya; the Center for Health, Human Rights and Development (CEHURD) in Uganda; and the Coalition for Women Human Rights Defenders (CWHRDs) in Tanzania. Publications and profiles from the African Women's Development and Communication Network (FEMNET), the Forum for African Women Educationalists (FAWE Africa) and other organisations were used only as contextual and supporting evidence.

The literature review identified six linked gaps that establish the rationale for the project: uneven understanding and use of the Universal Periodic Review (UPR) for SRHR advocacy; weak tracking and domestication of accepted recommendations; fragmented and issue-specific coordination; limited participation of grassroots, youth-led, disability-inclusive and other marginalized actors in decision-making; uneven organizational systems for monitoring, evaluation and learning (MEL), resource mobilisation and communications; and short-term, externally dependent financing. These documented gaps informed the baseline questions, stakeholder mapping and capacity-strengthening priorities.

2.4. Sampling Techniques

Purposive sampling was operationalized through partner-generated stakeholder longlists, country stakeholder mapping and the contextual knowledge of GEM Trust, CEHURD and CWHRDs. Eligibility required direct experience in SRHR movement building, advocacy, policy, service delivery, research, media, public administration or non-traditional ally engagement. Maximum variation was applied by country, actor type, organizational role, thematic focus and level of operation. Referrals were used to identify additional actors beyond partner networks, reducing gatekeeper bias and broadening inclusion.

Selection prioritized information-rich respondents and diversity rather than statistical representativeness. Potential replacements were drawn from the same stakeholder category when an invited respondent was unavailable. The analysis compared implementing-partner

perspectives with external civil society, government, media, professional and community perspectives and treated convergence, divergence and minority views explicitly.

2.5. Data collection Technique and Approach

Data collection used semi-structured KIIs, supplementary group work and validation discussions. Before participation, respondents were informed of the study purpose, intended use of findings, voluntary nature of participation, right to decline questions or withdraw, confidentiality arrangements and the separate choice to permit recording. Consent was obtained before each interview or group discussion. Personal data were minimised, securely stored and accessible only to the study team. In this revised report, respondents are described by broad stakeholder category and country; names and distinctive organisation-title combinations have been removed unless explicit publication consent was documented.

The attached country workbooks contain 56 distinct labelled KII records: 17 from Kenya, 17 from Uganda and 22 from Tanzania. The regional profile comprises 44 civil-society-affiliated respondents (12 implementing-partner staff and 32 representatives of other CSOs, networks or movements), 4 government or public-sector respondents, 6 media/private/professional or technical respondents, and 2 traditional leaders. Implementing-partner staff are therefore reported as a defined subcategory of civil society rather than as a separate overlapping category.

A) Key Informants with GEM Trust

In Kenya, a total of 17 key informant interviews were conducted that comprised of 3 implementing-partner staff, 11 representatives of other CSOs or networks, 2 media practitioners and 1 public-sector respondent. The attached group-work records from the Machakos capacity-building meeting were analyzed as supplementary Kenya evidence.

B) Key Informants with CEHURD

In Uganda, a total of 17 key informant interviews were conducted that comprised of 4 CEHURD staff, 9 representatives of other CSOs, coalitions or networks, 3 government/public-sector respondents and 1 technical or non-traditional respondent. The late-November Uganda engagement was incorporated as supplementary validation and group evidence.

C) Key Informants with CWHRDs

In Tanzania, a total of 22 key informant interviews were conducted that comprised of 5 implementing-partner staff, 12 representatives of other CSOs or networks, 2 traditional leaders and 3 media, health or legal/private-sector respondents.

2.6. Data Management and Quality Assurance

2.6.1. Analysis and Interpretation

Data management followed confidentiality, data-minimization and secure-access principles. Interview and group records were organised by country and respondent category, assigned controlled identifiers, and reviewed for completeness before coding. The revised analysis used a source register linking each finding to the relevant workbook, group record or documentary source without exposing the identity key in the report.

a) Qualitative Analysis

KII and supplementary group evidence were organised within a common thematic framework derived from the study objectives, tools and indicator matrix. Coding compared themes across countries and stakeholder categories, captured contrary and minority views, and linked each analytical claim to its source record. Country summaries were then triangulated with the desk review, approved application, inception design and monitoring and evaluation plan.

2.6.2. Data Quality Assurance

Quality assurance comprised review of the three country workbooks, removal of category overlap, reconciliation of country and regional totals, checks for duplicated respondent labels, comparison of findings against the qualitative-analysis reports, and verification of indicator wording against the monitoring and evaluation plan. Claims about piloting, backchecking or re-contact were retained only where supported by available records. The supplementary evidence was logged separately, coded consistently and included without inflating the verified KII total.

2.7. Ethical and Safeguarding Principles

2.7.1. Ethical Considerations

The baseline applied informed-consent, confidentiality, voluntary-participation and do-no-harm principles. Participants received an explanation of the study, intended use of information, recording arrangements, withdrawal rights and data-protection measures. The revised report removes direct identifiers and deductively identifying combinations of organisation and job title. Broad descriptors are used in the respondent profile and quotation labels, and any identity key is held separately from analytical files.

Sensitive information was stored using controlled access and password protection. Reporting was designed to minimize reputational, legal and psychosocial risk, particularly for actors working on abortion, sexuality education, LGBTQIA+ rights and other contested issues. Group evidence was used only at thematic level where individual consent and attribution permissions could not be verified from the attached records.

Any safeguarding concern identified during data collection was to be handled through the applicable EACHRights referral and escalation procedures. No participant was required to disclose personal experiences, and respondents could pause or stop participation. Findings

are presented in aggregate or anonymously, with limitations and evidentiary uncertainty stated transparently.

2.7.2. Social Inclusion and Integrating Gender Considerations

The study applied a gender-sensitive approach throughout its design and implementation. The team was gender-balanced, and data was disaggregated by age, gender, and disability. Sampling and analysis ensured inclusive representation and considered diverse perspectives across SRHR movements. Internal gender inclusion policies from EACHRights were reviewed and integrated into the process.

2.8. Limitations and Mitigations

The baseline used a mixed-mode approach. Most KIIs were conducted remotely, while the late-November Uganda engagement and the Kenya capacity-building meeting in Machakos provided in-person group and validation opportunities. In Tanzania, political tensions during the national elections delayed the start of data collection. However, once initiated, the team moved swiftly and efficiently resulting in a higher volume of completed interviews compared to Kenya and Uganda.

Mitigation measures included flexible scheduling, repeated follow-up, partner-supported mobilisation, supplementary in-person evidence collection and cross-country triangulation. Differences between individual interviews and group discussions were considered during interpretation. Remote recruitment created slower response and rescheduling among some government and senior civil-society actors but the in-person opportunities helped address coverage gaps and test the emerging findings.

CHAPTER THREE: BASELINE FINDINGS

3.1. Introduction

For clarity, key terms used in this chapter are written in full at first use: Sexual and Reproductive Health and Rights (SRHR), Universal Periodic Review (UPR), Civil Society Organisations (CSOs), Gender-Based Violence (GBV), Comprehensive Sexuality Education (CSE), Youth-Friendly Services (YFS), Lesbian, Gay, Bisexual, Transgender, Queer, Intersex and other diverse identities (LGBTQIA+), and Monitoring, Evaluation and Learning (MEL).

3.2. SRHR Legal and Policy Landscape

3.2.1 Regional Analysis

Regionally, the East African Community (EAC) Sexual and Reproductive Health Bill, 2021 proposes minimum standards for equitable access to SRHR information and services, including adolescent-responsive services. As at the report revision cut-off of 23 July 2026, the East African Legislative Assembly (EALA) document register still lists the instrument as a Bill rather than an enacted Community law. Non-passage would leave Partner States without a binding common SRHR standard, weaken regional accountability and perpetuate uneven national approaches.

While not a technical reality, advocates and media frequently label current sessions as a "last chance" to discuss the bill due to severe practical and political hurdles. Credible parliamentary procedures from EAC and civil society reports indicate that it is not legally true that this is the absolute "last chance" for the East African Community (EAC) Sexual and Reproductive Health and Rights (SRHR) Bill, nor is it guaranteed to never be presented again⁶.

Under East African Legislative Assembly (EALA) "Lapse and Reintroduction" rules, if an assembly term expires without passing a piece of legislation, the bill "lapses." However, this does not mean it can never return. Any Member of Parliament or committee in a subsequent assembly term can table a motion to reintroduce the bill as a fresh draft⁷.

Female genital mutilation/cutting (FGM/C) is also a cross-border SRHR and bodily-autonomy concern. Kenya prohibits FGM under the Prohibition of Female Genital Mutilation Act, 2011, and Uganda under the Prohibition of Female Genital Mutilation Act, 2010. Mainland Tanzania criminalizes FGM through Penal Code provisions introduced by the Sexual Offences Special Provisions Act, 1998, while the legal framework in Zanzibar is not equivalent. Across all three settings, enforcement, survivor support, cross-border movement and community acceptance remain key advocacy gaps.

⁶ <https://zamarafoundation.org/watering-the-eac-srh-bills-advocacy-through-traditional-media>

⁷ [East African Legislative Assembly](#)

In addition, EAC policy tools such as the *EAC Regional Reproductive, Maternal, Newborn, Child and Adolescent Health (RMNCAH) Policy Guidelines (2016–2030)*⁸ seek to guide Partner States in adopting integrated, rights-based approaches to reproductive health. The guidelines emphasize equity, quality of care, health-system strengthening, and improved monitoring and accountability. Equally important, regional frameworks, such as the *ESA Ministerial Commitment on Comprehensive Sexuality Education (CSE) and Sexual Reproductive Health (SRH) for Adolescents and Young People*,⁹ reinforce the political commitment of EAC states to expand adolescent access to comprehensive sexuality education and youth-friendly SRHR services.

Evidence from regional assessments, including the *2gether4SRHR baseline (2023/2025)*,¹⁰ shows that despite these policy instruments, alignment at the national level remains uneven, and the domestication of regional commitments faces challenges related to political resistance, resource limitations, and sociocultural norms.

Moreover, cross-border mobility and migration further shape the EAC's SRHR landscape. The *IOM SRHR, HIV and Migration Framework (2023–2027)*¹¹ highlights gaps in continuity of care for migrants and mobile populations and calls for coordinated regional approaches across the EAC, IGAD, SADC, and COMESA blocs. Migrant-inclusive SRHR policies, cross-border referral systems, and regional commodity and data harmonization are identified as priority areas for action.

Overall, the continental and regional SRHR landscape sets a strong normative and policy foundation, but its effectiveness depends on national implementation and civil-society advocacy. For SRHR movements in Kenya, Uganda, and Tanzania, these frameworks offer strategic entry points to demand accountability, harmonize policy across borders, and strengthen collective voice. The EAC instruments, particularly the SRHR Bill and the RMNCAH guidelines, are especially critical for advancing coordinated advocacy and ensuring consistency in policy and service delivery standards across the region.

3.2.2 Kenya

National Policies and Laws

Evidence from the desk review indicates that Kenya's SRHR policy and legal landscape is anchored in a progressive constitutional framework that guarantees the right to health, non-discrimination, and human dignity. *Article 43(1)(a) of the Constitution protects the right to the highest attainable standard of health, including reproductive health care*, while *Article 27 establishes equality and prohibits discrimination on grounds that are central to SRHR, such as sex, pregnancy, and disability*.¹² These constitutional provisions form the foundation for Kenya's policy environment on reproductive, maternal, and adolescent health.

⁸ [EAC Integrated Reproductive Maternal Newborn Child & Adolescent Health Policy Guideline \(2016-2030\)](#)

⁹ [UNESCO \(2013\)](#)

¹⁰ [2gether 4 SRHR \(2025\)](#)

¹¹ [IOM \(2023\)](#)

¹² [The Constitution of Kenya \(2010\)](#)

Kenya's commitments under regional and international instruments, such as the *Maputo Protocol*,¹³ *CEDAW*,¹⁴ and periodic reporting obligations, inform the national SRHR policy environment. The *Universal Periodic Review (UPR)* process continues to generate SRHR-related recommendations for Kenya, particularly around GBV, maternal health, harmful practices, and adolescent SRH. However, evidence shows persistent gaps in translating accepted UPR recommendations into actionable policy measures and budget commitments.

Findings further show that Kenya has developed multiple national frameworks that directly shape SRHR programming. These include the *Adolescent Sexual and Reproductive Health (ASRH) Policy (2015)*,¹⁵ the *National Reproductive Health Policy (2022- 2032)*,¹⁶ the *National Guidelines for Management of Sexual Violence (2014)*,¹⁷ the *National Gender and Development Policy (2019)*,¹⁸ and GBV-related legislative frameworks such as the *Protection Against Domestic Violence Act (2015)*.¹⁹ Evidence suggests that these instruments provide formal guidance on access to contraception, adolescent SRH information, GBV prevention, and maternal health, creating a legally supportive environment for SRHR delivery.

At county level, the proposed Kisumu County Sexual and Reproductive Healthcare Bill was publicly discussed in 2024 and is formally listed by the Kisumu County Assembly as the Kisumu County Sexual and Reproductive Healthcare Bill, 2026. As at 23 July 2026 it should therefore be described as a Bill, not an enacted law, and its budget implications remain subject to passage and implementation.

The Mombasa County Reproductive Healthcare Act, 2017 is a county law assented to on June 16, 2017, aimed at improving access to family planning, maternal care, and reproductive health information across Mombasa County. Key elements of the act focused on expanding the availability and affordability of family planning commodities and modern contraceptives and promoting progressive access to education on SRHR for youth and adolescents²⁰.

However, implementation of SRHR related policies is uneven across counties due to variations in county-level prioritization and resource allocation. The desk review findings shows that some policies at national level are outdated and require revision as with the ASRH Policy of 2015, largely because of the emerging dynamics, particularly around the issue of the age of consent. The KIIs also revealed that stakeholders frequently encounter inconsistencies in the interpretation and application of SRHR laws, particularly around abortion and adolescent access to services.

¹³ [AU Maputo Protocol on Women's Rights \(2003\)](#)

¹⁴ [CEDAW](#)

¹⁵ [Republic of Kenya National ASRH Policy \(2015\)](#)

¹⁶ [Republic of Kenya National Reproductive Health Policy \(2022- 2032\)](#)

¹⁷ [Republic of Kenya National Guidelines on Management of Sexual Violence \(2014\)](#)

¹⁸ [Republic of Kenya National Policy on Gender and Development \(2019\)](#)

¹⁹ [Republic of Kenya Protection Against Domestic Violence Act \(2015\)](#)

²⁰ [SRHR Alliance Kenya \(2019\)](#)

*“From a policy perspective, the frameworks exist, but counties interpret and prioritize SRHR very differently depending on leadership and resources.” – **KII, Public-Sector respondent, Kenya***

Data also suggests that certain dimensions of SRHR remain politically contested, especially abortion, LGBTQIA+ inclusion, and comprehensive sexuality education (CSE). While the Constitution permits abortion under specific circumstances, such as when a trained health professional determines that the life or health of the pregnant person is in danger, there remains inconsistent interpretation and enforcement of these provisions. Additionally, respondents also acknowledged the contradictory nature of various statutes which have seen a stalemate between the Constitutional provisions and other instruments such as the Penal Code and the Children’s Act.

The respondents engaged stressed the fact that on the one hand, the Constitution give the rights to access, but on the other hand, the Children’s Acts speaks to children under the age of 18 years not being able to access SRH services such as contraceptive services as well the prosecution of anyone who gives these services to minors. Policy reversals, court disputes, and conservative resistance continue to limit clarity and hinder service provision. CSE similarly faces significant opposition from religious and cultural groups, resulting in policy stagnation despite evidence supporting its public health benefits. Respondents also cited lack of political goodwill which often leads to policies being put at the back burner or being rejected. This points to a disconnect between legal instruments.

The findings established that the temporary withdrawal of The Ministry of Health’s 2012 Standards and Guidelines for Reducing Morbidity and Mortality from Unsafe Abortion in 2013 affected service delivery and access to safe abortion. The withdrawal of the guidelines were followed by a memo in February 2014 suspending the training of health workers on safe abortion care or the use of the drug *Medabon* for medical abortion. The Ministry threatened health providers with dire legal and professional consequences if they participated in any abortion training.

However, the High Court of Kenya ruled in 2019 that the withdrawal of standards and guidelines on abortion violated the rights to professional information, consumer protection and scientific progress. It also violated the right of women and girls to access information and the right to the highest attainable standard of health, which amounted to discrimination. The court further stated that since the Guidelines and the Training Curriculum are policy documents that had been developed through public participation, there should have been a similar procedure around their withdrawal²¹.

SRHR Budgeting and Financing

The desk review findings highlighted that Kenya’s SRHR financing environment is shaped by a mix of national policies and devolved budgeting processes, resulting in significant disparities in access and quality of services. Although the Constitution recognises the right to health, SRHR is not uniformly prioritised in county budgets. Many counties allocate minimal funds to adolescent SRH, GBV response, family planning commodities, and

²¹ [Article 19 \(2020\)](#)

community outreach, undermining implementation of national policies and weakening accountability.

At the national level, the transition from National Health Insurance Fund (NHIF) to Social Health Insurance (SHA) under the 2023 health reforms is expected to influence SRHR coverage, but the specific benefits package for reproductive health services remains unclear. Evidence shows that preventive and promotive SRHR services, such as CSE, family planning counselling, and adolescent-friendly services, are often underfunded or excluded from routine financing mechanisms.²²

The *Linda Mama Programme*,²³ which provides free maternal health services, has increased facility deliveries but does not cover the broader SRHR continuum, including post-abortion care, infertility, contraception, or GBV services. Stock-outs of family planning commodities remain common due to supply-chain challenges and procurement delays, further limiting access.

Discussions with key informants noted that service delivery gaps disproportionately affect adolescents, women living in informal settlements, persons with disabilities, and key populations. Barriers include provider bias, lack of youth-friendly services, limited facility accessibility for PwDs, and stigma toward LGBTQIA+ persons and unmarried adolescents. These barriers directly relate to baseline questions on inequalities, service access, and government responsiveness to SRHR-related UPR recommendations.

“Majority of young people from rural and marginalized communities are experiencing challenges in accessing quality SRHR service from public health care facilities due to inadequate youth friendly services. Many public health facilities and hospitals have not deployed adequate personnel who can support young people navigate access to SRHR that suits their needs” – KII with Implementing Partner

3.1.2. Uganda

National Policies and Laws

In Uganda, The National Objectives and Directive Principles of State Policy require the State to ensure access to health services and to protect women and children, while Article 21 guarantees equality and freedom from discrimination, Article 33 protects women’s rights, and Article 44 safeguards freedom from torture and cruel, inhuman or degrading treatment.

The findings also noted existence of laws that provide for specific SRHR issues like the Prohibition of Female Genital Mutilation Act No. 5 of 2010 and the HIV Prevention and Control Act, 2014. In the same measure the law criminalises certain aspects of SRHR in the Penal Code Act cap 120 including rape, defilement, incest, abortion, child to child sex, same sex acts, sex work and so many others

²² [Owino, Wangong'u, Were & Maleche \(2021\)](#)

²³ [Linda Mama Programme \(2025\)](#)

Policies that guide on access to SRHR services like the Health Sector Development Plan 2015/16–2019/2024, National Policy Guidelines and Service Standards for Sexual and Reproductive Health and Rights (2006), Consolidated guidelines for prevention and treatment of HIV in Uganda, 2016, Standards and Guidelines on Reducing Maternal Morbidity and Mortality from Unsafe Abortions in Uganda, 2015, and the Third National Development plan 2020/21 – 2024/25 all of which emphasise the provision of health services on for all on a non-discrimination basis.

There are also policies that guide access to SRHR information and services to adolescents like the National Adolescent Health Policy (2004), Adolescent Health Policy Guidelines and Service Standards (2012), National Youth Action Plan 2001, National Adolescent Health Strategy (2011-2015), National Guidelines on Health Education and Life Skills for Out-of-School Adolescents and Youth (OOSAY) 2026, Revised guidelines for the Prevention and Management of Teenage Pregnancy in School Settings in Uganda, 2020 and the National Strategy to End Child Marriage and Teenage Pregnancy 2022/2023–2026/2027. All of this address the SRHR of adolescents recognising that they need these health services and age-appropriate information to facilitate their development²⁴.

Despite the existence of these laws and policy frameworks, Uganda does not have a singular comprehensive law that addresses it in its entirety. The provisions are scattered and can only be found in different places, in fact sometimes the different documents contradict themselves which has made the implementation of the law and policy sporadic and uneven. For example, the age of consent to access contraceptives remains a point of contention with the 2006 SRHR guidelines allowing for adolescents of reproductive age to access short term contraceptives without parental or guardian consent but other policies demand for this consent.

Another example is on the laws that allow for access to safe and legal abortion which are the Constitution and the Penal Code Act only permit it on grounds to save the life of a pregnant woman, but the 2006 guidelines on SRHR in Uganda expand the grounds to include allowing abortion in cases of rape, defilement, and incest. This has caused confusion as to whether the expanded grounds in policy can be defended in court in case of an arrest. This calls for revisiting the existing laws, which often are archaic, and bringing them into alignment with the existing policies and international instruments to make sure they are in harmony with each other to facilitate easy access to SRHR for all²⁵.

The desk review findings further noted that the withdrawal of the Ministry of Health's specialized guidelines on safe abortion in Uganda in 2015 caused increased legal confusion, severe health risks for women, and provider intimidation. It left reproductive rights operating under a restrictive colonial-era penal code, heightening preventable maternal deaths and complications. The findings further noted that without clear operational guidelines, healthcare workers often feared criminal prosecution under the

²⁴ [Issue Paper Analysing Laws and Policies Affecting SRHR in Uganda: UGANET \(2022\)](#)

²⁵ [Issue Paper Analysing Laws and Policies Affecting SRHR in Uganda: UGANET \(2022\)](#)

Penal Code Act, leading them to refuse lawful care even when a mother's life or health is severely threatened²⁶.

SRHR Budgeting and Financing

The findings confirmed that Uganda's SRHR financing environment remains constrained by low public investment in health, with most reproductive health services dependent on donor funding. Domestic financing for key SRHR areas, family planning, adolescent SRHR, GBV response, and reproductive cancer services, remains insufficient, creating chronic gaps in commodity supply, infrastructure, and staffing. District-level planning and budgeting further reflect significant disparities, with rural and post-conflict regions experiencing greater shortages.

The respondents engaged noted that stock-outs of family planning commodities and essential SRHR medicines are common, and youth-friendly services are limited by inadequate trained staff, lack of privacy, and judgmental provider attitudes. While maternal health indicators have shown gradual improvement, the respondents highlighted that access to emergency obstetric care remains uneven, and post-abortion care services are often compromised by provider fear due to restrictive abortion laws.

*“Uganda faces significant SRHR financing and budgeting challenges, with government allocations to sexual and reproductive health often insufficient to meet demand and heavily dependent on external donors. For example, only a small portion of the health budget is earmarked for family planning commodities, and reductions in donor support have contributed to stock-outs of essential contraceptives, undermining service continuity for women and girls” – **KII, Implementing-Partner, Uganda.***

Integration of SRHR within Uganda's UHC agenda remains limited. Although SRHR services are included within the essential health package, resource constraints, competing priorities, and highly conservative sociopolitical dynamics restrict effective implementation. Key populations and LGBTQIA+ persons face additional barriers due to criminalisation and fear of harassment, which reduces service utilisation despite policy-level commitments.

These financing and service delivery gaps directly mirror the low implementation of SRHR-related UPR recommendations, many of which call for improved access, expanded service coverage, better protection for vulnerable groups, and enhanced health-system readiness. Without stronger and more consistent domestic financing, Uganda's ability to realise its SRHR commitments will remain limited.

3.1.3. Tanzania

National Policies and Laws

Tanzania's constitutional framework protects life, dignity and equality but does not expressly state a right to SRHR. The legal environment includes the Sexual Offences

²⁶ [Alexander Kagaha, Lenore Manderson: Power, Policy And Abortion Care In Uganda \(2020\)](#)

Special Provisions Act, 1998, the Law of the Child Act, 2009, the HIV and AIDS (Prevention and Control) Act, 2008 and Penal Code provisions relevant to abortion and same-sex relations. These instruments provide some protection from violence and discrimination while retaining significant restrictions affecting adolescents, women and sexual and gender minorities.

Mainland Tanzania's FGM prohibition derives from Penal Code amendments introduced through the Sexual Offences Special Provisions Act, 1998; it does not create an equivalent prohibition for Zanzibar. Enforcement gaps, cross-border movement and limited survivor-centred referral services remain important movement priorities.

The policy framework includes the Health Sector Strategic Plan V (2021–2026), One Plan III for reproductive, maternal, newborn, child and adolescent health, the National Adolescent Health and Development Strategy, adolescent- and youth-friendly service standards and implementation guides, and the Universal Health Insurance Act, 2023.

However, major contradictions arise in relation to adolescent SRHR, LGBTQIA+ rights, and access to safe abortion. Abortion is restricted under the Penal Code except when performed to save a woman's life, creating significant barriers to safe abortion and post-abortion care. Restrictions introduced in recent years, such as the 2017 directive banning pregnant girls and adolescent mothers from returning to school under President Magufuli, further entrenched discrimination, although recent reforms have begun reversing some of these policies.²⁷

Sexual and gender minorities face criminalisation under sections 154–157 of the Penal Code,²⁸ which undermines access to essential SRHR and HIV services, fuels stigma, and contradicts non-discrimination principles reflected in regional SRHR frameworks and UPR recommendations. Civic space has also been constrained by amendments to the *NGO Act*,²⁹ *Online Content Regulations*,³⁰ and *Media Services Act*,³¹ which limit freedom of association, assembly, and expression. These restrictions have direct implications on CSO-led advocacy, movement organising, and UPR engagement.

Human rights assessments highlight persistent violations including denial of services to adolescents, stigma toward unmarried women and key populations, and discriminatory treatment of sex workers, LGBTQIA+ persons, and people living with HIV. Enforcement of protective laws is uneven, and coordination across health, gender, and justice institutions remains weak.

Overall, Tanzania's constitutional and legal environment presents a restrictive landscape for SRHR, characterised by criminalisation, weak protections, and shrinking civic space. These challenges significantly limit implementation of SRHR-related UPR

²⁷ [Human Rights Watch \(2022\)](#)

²⁸ [SIDA \(2014\)](#)

²⁹ [THE NGO Act of 2023](#)

³⁰ [Tanzania Electronic and Postal Communications \(Online Content\) Regulations \(2020\)](#)

³¹ [Tanzania Media Services Act of 2023](#)

recommendations and require social movements to adopt adaptive, risk-aware, and coalition-based advocacy strategies.

SRHR Budgeting and Financing

The desk review findings established that Tanzania's SRHR financing environment is constrained by persistently low public investment in health, with government allocations remaining far below the Abuja target and SRHR funding largely absorbed into broader RMNCAH budgets. The National Adolescent Health and Development Strategy notes that no dedicated budget exists for adolescent SRHR and that health sector allocations have remained at approximately 7% of the national budget, limiting the scale and sustainability of key interventions.³²

These gaps are compounded by declining external aid and the country's continued reliance on development partners for essential commodities and HIV services. Out-of-pocket payments account for a substantial proportion of total health spending, limiting access for adolescents, who remain largely uninsured and face financial barriers to care.³³ Although the *Universal Health Insurance Act of 2023* seeks to harmonize fragmented insurance schemes and expand coverage, implementation remains in early stages, and financial protection mechanisms are still weak and uneven.³⁴

These systemic constraints contribute to frequent stock-outs of contraceptives and SRHR commodities, limited adolescent-friendly services, staff shortages and disparities in service readiness between urban and rural facilities. Stigma and restrictive norms further limit access for AGYW and key populations, resulting in an SRHR service environment that remains inequitable and insufficiently responsive.

3.2 Baseline Indicators Analysis

Indicator 1: AmplifyChange grantee undertakes rigorous advocacy planning and/or regular advocacy plan reviews for SRHR policy change.

Engagement of SRHR social movements in coordinated advocacy using the UPR processes to advance the SRHR agenda

Kenya: The baseline study revealed that SRHR social movements serve as coordination platforms that unite diverse civil society organizations under shared advocacy agendas. Within these formations, members identify priority issues, harmonize strategies, and avoid duplication of effort by sharing resources, information, and networks. This synergy enhances the collective strength of the movement and improves engagement with state and non-state actors. The process of convening and co-convening thematic working groups, such as under the UPR mechanism, ensures strategic alignment and amplifies collective voice in policy spaces.

³² [Tanzania Ministry of Health, Community Development, Gender, Elderly & Children \(2018\)](#)

³³ [Tanzania Ministry of Health, Community Development, Gender, Elderly & Children \(2018\)](#)

³⁴ [Tanzania Universal Health Insurance Act, 2023 Policy Brief \(2024\)](#)

“Most of the civil society organizations under the UPR come together and we usually have the organizational updates. We learn that someone will speak about the RH policy, and another one will speak on something else they're working on. So, it helps come up with priority issues that we need to address.”

KII with Representative from GEM Trust

The findings also revealed that the UPR process has become a vital entry point for advocacy on sexual and reproductive health and rights in Kenya and the region. The respondents highlighted how engagement in the UPR has deepened their understanding of human rights accountability and provided structured mechanisms for influencing government commitments. Through participation in mid-term and full UPR cycles, organizations have learned to strategically package SRHR issues and track progress on implementation.

“My experience in the UPR has shown me that having a well-coordinated effort when it comes to advocacy is key. I got to understand how to network better, how to communicate better, how to package information more effectively. It's not just the knowledge gained but also the skills polished in the process. My experience within the UPR has been eye-opening because I got to understand what the UPR is and its significance. This was through a joint initiative between GEM Trust and EACHRights.”

KII with Representative from GEM Trust

Uganda: The study findings revealed that the UPR mechanism is considered an important tool for advocacy by SRHR social movements because it provides a structured avenue for governments to make commitments on human rights, including SRHR, which social movements can then use to hold them accountable. The UPR helps formalize expectations, ensuring that recommendations are documented and that governments can be monitored for follow-through. It also raises the visibility of SRHR issues, enabling movements to use internationally recognized recommendations as leverage for advocacy at national and regional levels.

“It's effective because at least we get our governments to make commitments that we can hold them accountable for. Most of them, when they come back, at least you see the different line ministries trying to work towards fulfilling the recommendations that were given. Also, ensuring that the commitments are popularized and people get to know what they committed to and how they are going, and what they are going to do towards ensuring that at least the health of the population improves.”

KII with Implementing Partner, Uganda

The study findings also revealed that the UPR mechanism serves as a critical means for identifying gaps in SRHR service provision and highlighting challenges in accessibility, affordability, and acceptability. It also functions as a resource mobilization tool: global SRHR movements and development partners use UPR findings to guide funding decisions, particularly in low- and middle-income countries. By exposing areas where governments fall short, UPR reports help movements target advocacy efforts and strategically engage both national and international actors to address resource gaps and policy weaknesses.

“The UPR process gives opportunity for the global SRHR movements to mobilise resources for low and middle-income countries. It is important that it is held, but also it helps to highlight gaps in SRHR in terms of service availability, accessibility, in terms of affordability, but also acceptability. It brings in issues to deal with legal regimes and how countries are performing, and what should be done.”

KII with Representative from the Coalition for Health Promotion and Social Development

Tanzania: Civil society and SRHR movements in Tanzania have actively engaged in the UPR process by building capacity, reviewing state human rights reports, and generating alternative evidence to hold the government accountable for sexual and reproductive health and rights commitments. For example, CWHRD conducted a two-day training on Gender and Reproductive Health and the UPR system in 2025 that enabled participants to critically assess and verify government reporting on reproductive and gender health issues, including SRHR concerns³⁵.

The study findings also highlighted that broader civil society coalitions coordinated by organizations like the Legal and Human Rights Centre (LHRC) and others have organized multi-stakeholder consultations and compiled shadow reports to the UN Human Rights Council, amplifying SRHR priorities within the UPR dialogue alongside other rights issues. A joint mid-term review report by some 228 CSOs, facilitated by LHRC and partners, reflects sustained monitoring and advocacy around the implementation of accepted UPR recommendations, including those with implications for reproductive health and rights.³⁶

Increased receptiveness to SRHR-related recommendations by government stakeholders as a result of CSOs' advocacy action

Uganda: The study findings established that CSOs in Uganda are actively engaging in SRHR advocacy within the UPR process by building networks, generating evidence, and pushing for accountability on sexual and reproductive health and rights commitments. Groups such as CEHURD and *SRHR Alliance Uganda* provide platforms that bring together youth-led, women's, key population, and disability-inclusive organisations to harmonize advocacy agendas and strengthen collective voices around national and international commitments, including UPR follow-up actions on SRHR issues.

However, the study revealed that a major challenge in pushing for SRHR reforms through the UPR process is the limited political will and prioritization by government actors. While recommendations are acknowledged, there is often reluctance to fully commit to implementation, limiting the effectiveness of advocacy efforts. The findings also revealed that challenges also arise from the misalignment between UPR recommendations and national development priorities. Parliamentary committees may deprioritize SRHR reforms

³⁵ [CWHRDS \(2025\)](#)

³⁶ [Save The Children \(2021\)](#)

if they do not feature in the government's development plan, and limited historical funding for these programs further constrains advocacy efforts.

“One of the challenges that we faced in pushing was when we were engaging with the parliamentary health committee. If you have a national development plan that does not prioritize UPR processes, it becomes hard. The parliamentarians will tell you this is not part of their priority for the government in the next five years”

KII with Representative from Strong Minds Uganda

Tanzania: The desk review findings established that CSOs in Tanzania have helped create greater receptiveness among government stakeholders to SRHR-related UPR recommendations through sustained advocacy, evidence generation, and multi-stakeholder engagement. The findings revealed that coordinated advocacy platforms and national dialogue processes such as the Collaborative Advocacy Action Plan (CAAP) involving the Ministry of Health and partners have translated SRHR-related commitments into concrete plans for adolescent-friendly services, accountability frameworks, and community engagement goals, indicating increased government willingness to work with civil society on SRHR priorities.³⁷

However, the findings also established that face persistent challenges in convincing the government to adopt and fully implement SRHR-related UPR recommendations, largely due to political sensitivity around sexual and reproductive rights. The desk review findings revealed that nearly half of the SRHR-related recommendations Tanzania received in its UPR 2021 were rejected by the government,³⁸ reflecting resistance to addressing issues such as discrimination, access to health services for key populations, and legal reforms for women's equality which undermines CSOs' advocacy efforts.

Kenya: The study findings highlighted that government stakeholders have shown increased receptiveness to SRHR-related UPR recommendations as a result of concerted CSO advocacy actions, with organizations such as EACHRights, GEM Trust, KELIN and the broader Right to Health Thematic Group engaging ministries and state agencies in sustained dialogue following the country's 4th UPR cycle in 2025. After submitting comprehensive civil society stakeholder reports and analysing the government's UPR responses on health and SRHR issues, CSOs have convened multi-stakeholder forums with the Ministry of Health and other duty bearers to clarify commitments, identify gaps in SRHR implementation, and collaboratively develop strategies for follow-up action including discussions on improving access to SRHR services for adolescents, key populations, and people with disabilities.

“These engagements demonstrate a shift toward greater government engagement and acknowledgment of SRHR challenges, as seen when state officials, such as the Director of SRHR at the Ministry of Health, participated in joint CSO-government accountability

³⁷ [PMNCH \(2025\)](#)

³⁸ [UPR Report Tanzania \(2021\)](#)

meetings and expressed openness to collaborative approaches to address financing, access barriers, and inefficient service delivery”

KII with Ministry of Health in Kenya

Level of Implementation of SRHR-related UPR recommendations by the governments of Kenya, Uganda and Tanzania

Kenya: The desk review findings established that the government of Kenya has adopted or made commitments to implement SRHR-related recommendations include commitment to expand and improve access to sexual and reproductive health services, including strengthening maternal health care, family planning, emergency obstetric care, and public awareness of SRHR, especially for young people and other underserved groups. Kenya accepted these recommendations during its 4th cycle UPR review and reaffirmed them at adoption at the Human Rights Council’s 60th session.³⁹

Kenya also committed to strengthen its HIV response and pursue universal health coverage, which includes protecting and promoting SRHR and ensuring access to necessary sexual and reproductive health services.⁴⁰ However, recommendations relating to safe abortion, adolescent autonomy, comprehensive sexuality education, and non-discrimination of LGBTQIA+ persons remain largely unimplemented, with government citing cultural, religious, and political sensitivities.

Civil society assessments, including the UPR Kenya SRHR Thematic Submission (2024), show that implementation is uneven and often limited to policy statements rather than concrete action. There is no comprehensive national mechanism dedicated to tracking SRHR-related UPR recommendations, and engagement is frequently ad-hoc rather than institutionalised. Key government duty-bearers, such as MoH, NGECC, and KNCHR, have varying levels of awareness and commitment, and county governments are rarely involved despite being responsible for service delivery.

Uganda: The findings revealed that Uganda’s government has received a wide range of SRHR-related UPR recommendations across successive cycles, including on GBV prevention and response, reducing maternal mortality, eliminating harmful practices, addressing unsafe abortion, expanding adolescent SRHR, protecting the rights of persons with disabilities, and ensuring non-discriminatory access to health services.

Examples of SRHR-related recommendations accepted by Uganda in past UPR cycles include increase access to sexual and reproductive health services, including by raising the health budget to the Abuja Declaration target of 15 percent, which explicitly included SRHR services such as family planning and maternal health care,⁴¹ continue working towards reducing high maternal mortality and increasing access to sexual and reproductive health services, reflecting commitments to improve reproductive health outcomes for women and girls.⁴²

³⁹ [Sexual Rights Initiative \(2025\)](#)

⁴⁰ [KELIN \(2025\)](#)

⁴¹ [Center for Reproductive Rights \(2022\)](#)

⁴² [OHCHR.ORG 2024](#)

“I have heard about and participated in the UPR processes, and I know Uganda made some commitments and they have to report. Unfortunately, sometimes they report when it is too late or sometimes, they government reports do not present a true reflection of what happens in the ground. This is because of limited participation of government in CSO led UPR shadow reporting processes.”

KII with Representative from Uganda Youth and Adolescents Health Forum (UYAHF)

However, recommendations relating to decriminalisation of same-sex relations, CSE, and access to safe abortion remain consistently noted or only partially accepted. The findings also noted that shadow reports and UPR stakeholder submissions consistently highlight that most SRHR-related recommendations remain partially implemented or not implemented at all, primarily due to restrictive legal frameworks, political resistance, and inadequate financing. There is no dedicated national mechanism for monitoring the implementation of SRHR-related UPR recommendations, and awareness among government duty bearers is uneven across ministries and districts.

Civil society actors, particularly CEHURD and other SRHR coalitions, play an active role in preparing joint submissions and monitoring progress, but their engagement is often constrained by a shrinking civic space, limited access to policy dialogues, and risks associated with advocacy on sensitive SRHR issues, most notably LGBTQIA+ rights and abortion. Government participation in CSO-led UPR forums is irregular, and there are few structured opportunities for joint planning or accountability review.

Tanzania: Tanzania has received repeated UPR recommendations calling for strengthened GBV prevention and response, improved access to SRHR services, protection of key populations, and harmonization of national laws with human rights commitments.⁴³ The findings established that recent SRHR-related UPR recommendations that Tanzania accepted during its 39th UPR cycle (reviewed in November 2021 and adopted at the 49th Human Rights Council session in March 2022) included strengthening access to health services, protecting the rights of women and girls (including maternal health), and upholding gender equality as part of its broader human rights commitments.

In its recent review, Tanzania agreed to implement numerous recommendations aligned with advancing SRHR, such as aligning laws and policies to strengthen reproductive health services, reduce maternal mortality, and promote gender equality within health and education systems. These were accepted as part of the set of 167 recommendations to be implemented. The findings further highlighted that review statements from Tanzania’s UPR process highlighted the acceptance of recommendations to ensure pregnant school-aged girls can continue their education

⁴³ [Tanzania CSOs UPR Joint Civil Society Submission on Human rights Concerns in Tanzania \(2021\)](#)

which is a key SRHR-related issue affecting adolescent reproductive rights and access to services.⁴⁴

However, while the government has accepted many recommendations related to eliminating violence against women and children and improving maternal health, key informants with partners in the SRHR sector noted that progress remains uneven due to restrictive laws, underfunding, and weak coordination. Civil society UPR submissions highlight continuing barriers to access to contraception, comprehensive sexuality education, and adolescent-friendly services, alongside the persistent criminalization of sex work and same-sex relations, which limits service uptake and undermines non-discrimination commitments.

The NPA-VAWC II outlines multi-sector actions aligned with UPR priorities, yet subnational implementation is slow, and referral mechanisms remain inconsistent.⁴⁵ Limited data systems, weak monitoring of UPR follow-up, and restricted civic space further constrain alignment, resulting in low visibility of SRHR issues within formal state reporting. As a result, progress toward UPR recommendations on access, equity and protection of vulnerable groups remains limited, despite strong policy commitments on paper.

Indicator 2: Increased CSO participation in policy and budgetary processes

Advocacy actions and strategies taken to engage duty bearers on SRHR-related UPR recommendations

Kenya: The study findings revealed that organizations and CSOs like EACHRights actively engage in policy-level advocacy to influence SRHR-related legislation and national guidelines with a key focus on ensuring that SRHR policies are aligned with best practices and national priorities, including budget allocations. Stakeholder platforms such as META Kenya provide a structured space for organizations to lobby legislators, identify key policy gaps, and collaborate with government departments, including the Ministry of Health. These engagements often involve both individual and collective efforts to drive policy reforms that enhance access to SRHR services and commodities across the country. The findings noted that GEM Trust, through the SRHR Thematic Group has helped foreground recommendations that call for reviewing legal and policy barriers to SRHR access for adolescents, young people, persons with disabilities, people living with HIV and key populations, as part of Kenya's UPR stakeholder submissions and follow-up advocacy.

“Key advocacy initiatives are mainly on SRHR-related policies and in budget advocacy at the national and county levels. When we organize these stakeholder engagements, the purpose is to ensure we have organized efforts to influence policy or practice, and we lobby with the legislators”

KII with Representative from Access to Medicines Platform (ATMP)

⁴⁴ [The Citizen+1 2022](#)

⁴⁵ [Tanzania National Plan to End Violence Against Women & Children \(2024/25- 2028/29\)](#)

The study findings also noted that CSOs and organizations generate evidence on the availability, affordability, accessibility, and quality of SRHR services to inform policy dialogue and campaigns. The research conducted not only underpins budget advocacy and policy reviews but also strengthens the legitimacy of advocacy demands. By leveraging empirical data, organizations can tailor interventions to county- or national-level challenges, ensuring advocacy efforts are evidence-based and contextually relevant. Further, study findings confirmed that in contexts where SRHR initiatives face strong social, cultural, or political resistance, organizations often turn to legal advocacy as a primary strategy. Litigation serves as a mechanism to enforce constitutional and policy mandates, especially on contentious issues such as comprehensive sexuality education, LGBTQI+ rights, and reproductive health access.

Tanzania: The study findings reported that CSOs like WGNRR engage in SRHR advocacy by pursuing legal reform and strategic litigation, including initiating a High Court case to clarify the law and domesticate the Maputo Protocol in Tanzania. Their approach combines grassroots capacity building, coalition work, and the development of advocacy briefs to address gaps in re-entry policies and unsafe abortion. CWHRDs advances UPR-aligned SRHR goals by organizing dialogues and capacity-building on unsafe abortion, post-abortion care and women's health rights, recommending measures to protect women and girls and strengthen SRHR service provision at community and policy levels

The findings also confirmed that Sauti Ya Wanawake have been conducting SRHR advocacy at local, national, and regional levels that focuses on enabling adolescent girls to continue schooling after pregnancy, ensuring access to menstrual hygiene products, and improving health facility infrastructure. These initiatives reflect both systemic challenges and grassroots engagement in advancing SRHR and gender equity in Tanzania.

“Advocacy is also part of our work at Sauti Ya Wanawake. We supported advocacy on re-entry education policy, access to menstrual pads and lobbying the government to provide renewable energy to health facilities.”

KII with Representative from Sauti Ya Wanawake Ukerewe

Uganda: The study findings revealed that SRHR advocacy by organizations like AHF Uganda is deeply integrated into programmatic work, spanning national and local engagement, policy influence, and community-level empowerment. Advocacy initiatives cover a broad spectrum, including engagement with government ministries, networks, and platforms dedicated to reproductive, maternal, child, and adolescent health (RMNCAH).

The findings also highlighted that CEHURD is pushing for increased government investment in SRHR financing, stronger family planning and adolescent health services and accountability for equitable reproductive health access in national budgets, emphasizing expanded support for maternal health and reproductive care as part of its public budget advocacy and human rights work. Organizations strategically partner with CEHURD to leverage complementary expertise and co-advocate on complex issues like comprehensive sexuality education and adolescent SRHR frameworks.

CSOs like RHU engage in SRHR advocacy through participation in legislative reviews, policy development, and national strategic planning. Their efforts include involvement in

the Sexual Offences Bill Review, the East African Community FGM and SRHR Bill, and the ADH Costed Implementation Plan for adolescents and young people. RHU also participates in developing educational frameworks and project-level initiatives like Fireside Talks targeting youth, while engaging in regional and international advocacy to amplify SRHR issues.

“We participated in the review of the Sexual Offences Bill , the EAC FGM and SRHR Bill processes and development of national guidance on health education and life skills for out-of-school adolescents and youth”

KII with Representative from Reproductive Health Uganda

SRHR-related law, policy, and budget reforms in Kenya, Uganda, and Tanzania influenced by CSOs

Kenya: The study findings highlighted that CSOs like EACHRights and coalitions like the Kenya SRHR Alliance have partnered with government and stakeholders to push for youth-friendly services, comprehensive sexuality education, and policy engagement at both national and county levels, strengthening CSO voices in decision-making forums and advocating for supportive regulations and budget lines that reflect community SRHR priorities.

Desk review findings also confirmed that Groups such as Marie Stopes Kenya⁴⁶ have successfully advocated for inclusion of key reproductive health guidelines like the *combi-pack* for medical abortion in the national essential medicines list and contributed to the development of broader health policy tools including *Self-Care Guidelines* and Universal Health Coverage-related laws, demonstrating how sustained civil society pressure can shape the legal and operational environment for SRHR services.

The findings noted that entities like the Kenya SRHR Alliance supports county-level budget advocacy and policy engagement to ensure SRHR priorities are included in public financing decisions, strengthening CSO influence over resource allocation for services and commodities.⁴⁷ CSOs like Youth Alive! Kenya have trained youth advocates to present memoranda to county health authorities and influence integrated development plans and budgets to reflect family planning and reproductive health needs, and platforms such as the Access to Medicines Platform (AtMP) and local CSO networks have conducted SRH budget training and evidence dissemination workshops, equipping grassroots organizations to engage effectively in county budget cycles and advocate for increased SRH and GBV funding.⁴⁸

Tanzania: The study findings highlighted that coalition efforts involving the Sexual and Reproductive Health Rights (SRHR) Coalition and the Doris Mollel Foundation have publicly urged the government to adopt gender-sensitive budgeting for health services,

⁴⁶ [Marie Stopes Kenya \(2024\)](#)

⁴⁷ [Kenya SRHR Alliance](#)

⁴⁸ [Access to Medicines Platform](#)

including resources for maternal and newborn care and community health infrastructure.⁴⁹

The findings also established that national advocacy efforts such as Tanzania's Collaborative Advocacy Action Plan (CAAP), developed through consultation between the Ministry of Health and multi-stakeholder partners, reflect how civil society engagement has helped shape joint accountability frameworks and priority commitments for adolescent-friendly SRH services and increased health financing.⁵⁰

Organizations like CWHRD and Teens Corridor continue to build grassroots movements advocating for legal reforms on issues such as child marriage and reproductive health norms, indicating sustained pressure from CSOs to translate SRHR advocacy into law and policy change.

Uganda: The findings revealed that CSO participation in policy and budget processes is organised through coalitions and structured engagement with duty bearers. Reproductive Health Uganda, the SRHR Alliance Uganda, the Civil Society Budget Advocacy Group and allied organisations contributed to health-sector positions for the FY 2025/26 budget and district family-planning costed implementation plans. CEHURD also convenes quarterly update meetings with government ministries, departments and agencies (MDAs), alongside periodic engagement with Members of Parliament, particularly the Health Committee, to identify SRHR challenges and develop multi-sectoral responses.

Commitments by Legislators, regulators, and other decision-makers that demonstrate efforts towards advancing SRHR

Tanzania: The study findings revealed that legislators and other decision-makers in Tanzania have demonstrated growing commitments toward advancing SRHR through policy and strategic plans that recognize the importance of sexual and reproductive health as part of national development goals. The findings highlighted that organizations like CAAP and CWHRD have been engaging in high level advocacy in partnership with Ministry of Health and partners to develop strategies that outline national targets to strengthen adolescent-friendly services, expand youth SRHR education, enhance neonatal care, and increase health financing including commitments to improve access and quality of SRH services by 2030.

“Our governments engagement with international frameworks such as the FP2030 family planning commitments shows parliamentary and executive support for policies aimed at increasing contraceptive access and domestic resource allocation for reproductive health commodities as part of long-term health planning. These actions reflect an evolving policy environment in which SRHR is increasingly integrated into national strategies and legislative priorities”

KII with Representative from Macho Kwa Jamii

⁴⁹ [The Guardian News 2025](#)

⁵⁰ [CAAP 2024](#)

Despite formal commitments by lawmakers and decision-makers to advance sexual and reproductive health and rights, the study findings established that significant implementation challenges like deep-seated social stigma, cultural norms, and limited societal acceptance of adolescent SRHR services continue to restrict young people's access to comprehensive sexuality education and youth-friendly services even where policies nominally exist. For example, stigma and lack of age-appropriate information remain major obstacles to improving adolescent SRHR outcomes, slowing the translation of policy into practice.⁵¹

Kenya: The desk review findings revealed that Kenyan legislators and decision-makers have shown renewed commitment to advancing sexual and reproductive health and rights (SRHR) through both policy engagement and public advocacy in 2024–25. At the national policy level, the Ministry of Health has set ambitious *FP2030 family planning targets* aimed at increasing contraceptive prevalence and reducing adolescent pregnancy rates, which demonstrates government prioritization of reproductive health outcomes in policy planning.⁵²

The findings also noted that strategic collaborations such as the Memorandum of Understanding between the National Council for Persons with Disabilities and MSI Reproductive Choices Kenya reinforce decision-maker commitments to inclusive SRHR services for persons with disabilities, reflecting a broader legislative and administrative effort to ensure rights-based access to reproductive health care.⁵³

Other key commitments made by legislators demonstrating efforts on SRHR include education-sector linkage through the back-to-school policy indicator for girls, tied to protection of pregnant/parenting learners and reducing dropouts and development of the National Guideline for Self-Care in Reproductive Health to expand access via regulated self-care interventions (family planning, maternal health, STI services, reproductive cancers, etc.) and support county domestication.

“There have been some efforts by legislators, especially the women representatives in submitting proposals and legislative agendas on SRHR in parliament and senate. However, this needs to be backed up in national and county level budgets. Legislative commitments and actions proposed can only be relevant if resources and clear budget lines are allocated to support service provision”

KII with Representative from Plan International

However, the study findings also noted that deep-rooted religious and cultural opposition to aspects of SRHR, such as comprehensive sexuality education and contraceptive use, continues to influence policy enforcement and deter public officials from fully backing progressive reforms, limiting practical implementation of laws and commitments.

⁵¹ [UNFPA Tanzania 2024](#)

⁵² [Ministry of Health 2024](#)

⁵³ [National Council for Persons with Disabilities 2025](#)

For example, SRHR advocates during the survey reported that restrictive national laws and political reticence fueled by moral beliefs hinder service provision and discourage providers and young people from accessing essential services. The respondents highlighted that inadequate allocation of domestic resources such as the 2025/26 family planning budget being well below estimated needs and uneven implementation of adolescent and youth SRH policies demonstrate gaps between legislative intent and on-the-ground delivery, with county-level funding and awareness shortfalls further weakening impact.

Uganda: Legislative, regulatory, and institutional leaders in Uganda have demonstrated efforts toward advancing Sexual and Reproductive Health and Rights (SRHR) through multi-sectoral programs, policy guidelines, and budget commitments led by key national bodies. Notable actions include the Ministry of Health launching initiatives, Parliamentary orientation platforms, and localized Family Planning 2030 financing goals.

The findings noted that the Government of Uganda commits to annually ring fence 50% of the domestic resources allocated for procurement, warehousing and distribution of FP commodities from the reproductive health (RH) commodities budget (NMS Vote 116 under Output 15- Supply of Reproductive Health Items) by 2025.

The Ministry of Health launched the SAY Plus+ Programme which is a USD 50 million initiative (running 2024–2030) alongside international partners to scale up adolescent and youth SRHR access across 13 underserved districts. The Ministry also developed the National Policy Guidelines and Service Standards to integrate reproductive health services and lower maternal/infant mortality in line with Sustainable Development Goals.

However, discussions with CSOs and SRHR actors indicated that restrictive laws such as those criminalizing abortion except to save a woman's life create ambiguity and deter healthcare providers from offering comprehensive reproductive services, particularly for marginalized groups like sex workers, undermining SRHR policy intent. This legal confusion, coupled with strong religious and cultural stigma around contraception and adolescent sexual health, limits access to services and hinders legislative commitments from yielding real change.

Indicator 3: AmplifyChange grantees strengthen their own organizational capacity

Existing organizational systems and practices on daily operations

GEM Trust: The study revealed that GEM Trust provides sustained technical support to CBOs, particularly those operating in informal settlements and underserved regions. Its approach is grounded in feminist principles, emphasizing safety, dignity, and agency. One of its core contributions is building the capacity of grassroots actors to document and advocate for SRHR using ethical storytelling and rights-based framing. This includes training WHRDs and youth-led groups on how to record lived experiences of SRHR violations such as denial of post-abortion care, stigma around adolescent sexuality, and gender-based violence and translate them into policy-relevant narratives.

GEM Trust also supports CBOs in preparing UPR submissions and shadow reports, ensuring that community-level data informs national and international advocacy.⁵⁴ In addition, it equips organizations with media engagement strategies, helping them challenge harmful portrayals of women and girls in mainstream media and promote accurate, empowering narratives around SRHR. This technical support is embedded in long-term partnerships that prioritize grassroots leadership and movement sustainability.

Further examination of available documentation shows that GEM Trust stands out as one of the most institutionally robust partners. The due diligence assessment underscores GEM Trust's strengths in governance, safeguarding, financial management, and MEL, which enable it to coordinate multi-stakeholder initiatives and anchor national advocacy platforms such as the *UPR Kenya Stakeholder Coalition* and the *Coalition Against Sexual Violence*. Its capacity to convene diverse actors, generate evidence, and influence national dialogues positions it as a central pillar in Kenya's SRHR landscape.

CEHURD: A review of organizational documents revealed that CEHURD plays a central role in convening SRHR actors across legal, health, and community sectors. It leads national coalitions such as the Coalition to Stop Maternal Mortality Due to Unsafe Abortion (CSMMUA) and contributes legal expertise to the SRHR Alliance Uganda, helping shape national reproductive justice agendas.⁵⁵ Its convening power is especially visible in strategic litigation networks, where CEHURD brings together lawyers, health professionals, and civil society organizations to challenge laws and policies that restrict access to SRHR services.

CEHURD provides sustained technical support to grassroots organizations, youth platforms, and health advocates. It trains community-based organizations and youth-led groups on legal literacy, rights-based advocacy, and documentation of SRHR violations,⁵⁶ equipping them to engage with district leadership and national policy platforms. Its technical support also extends to strategic litigation capacity-building, enabling local actors to understand and navigate Uganda's legal system in defense of health rights. CEHURD also produces legal research and policy briefs that inform national debates and legislative processes, positioning itself as a legal backbone within Uganda's SRHR movement. Its framing ensures that SRHR violations are not treated as isolated incidents but as systemic injustices requiring legal remedy.

CWHRD: The study findings confirmed that CWHRDs plays a critical convening role by bringing together women defenders from diverse movements, including SRHR, LGBTQIA+ rights, land rights, and gender-based violence, into a unified platform for protection and advocacy. It facilitates national and regional dialogues, often in collaboration with the Tanzania Human Rights Defenders Coalition (THRDC) and the East Africa Women Human Rights Defenders Network (EAWHRDN), to strengthen collective voice and visibility⁵⁷

⁵⁴ [GEM Trust - UPR Kenya Stakeholder Presentations](#)

⁵⁵ [SRHR Alliance Uganda - Member Organisations](#)

⁵⁶ [CEHURD - Community Engagement Reports](#)

⁵⁷ [THRDC - Strategic Plan and Partnerships](#)

The coalition provides capacity strengthening and protection support to its members and allied organizations. This includes training on digital and physical security, trauma-informed advocacy, and documentation of threats and violations. CWHRDs also supports grassroots feminist groups, particularly those working on SRHR, safe abortion, and post-abortion care, to navigate legal risks and engage in rights-based advocacy.⁵⁸ Its technical support is grounded in feminist care ethics, ensuring that defenders are not only equipped to advocate but also supported to heal and sustain their activism.

CWHRDs frames SRHR as a feminist and political struggle for bodily autonomy, dignity, and freedom from violence. It challenges the criminalization and stigmatization of women defenders who advocate for safe abortion, LGBTQIA+ rights, and adolescent sexuality. Through its advocacy, CWHRDs exposes how legal and cultural systems silence women's voices and restrict access to SRHR services. It also documents violations against WHRDs and uses this evidence to engage with national human rights institutions and international mechanisms.

Organizational Capacity Needs

The baseline organizational-strengthening priorities derived from the Due Diligence, Organisational Capacity Assessment and approved monitoring and evaluation workplan highlighted key areas of capacity strengthening for the 3 implementing organization from Kenya, Uganda and Tanzania.

For GEM Trust, the priorities are evaluation and renewal of the strategic plan, registration with the data-protection authority, staff training on organizational policies, and capacity strengthening for the board, management and staff on fundraising and resource mobilisation.

For CEHURD, the priorities are stronger institutional annual reporting, staff policy training, data-protection registration, development of an advocacy strategy and communications policy, and migration from traditional QuickBooks to an integrated enterprise resource planning system.

For CWHRDs, the priorities are governance and leadership training for new board members, a management succession plan and staff training on organizational policies. Across partners, annual financial audits and project audit arrangements provide a common accountability baseline.

Indicator 4: Partnerships established with non-traditional SRHR allies

Capacities of Non-traditional SRHR allies to advocate and engage more effectively with duty bearers on SRHR-related UPR recommendations.

The study findings highlighted that across Kenya, Uganda, and Tanzania, non-traditional SRHR allies like media, activists, faith leaders, and traditional leaders are building capacities that strengthen their ability to advocate and engage duty-bearers on SRHR

⁵⁸ [VCSA Fund – Partner Profile: CWHRDs Tanzania.](#)

issues including within UPR processes. In Kenya, organisations such as Youth Changers Kenya have trained religious leaders, journalists, and policymakers using an *Intersectional SRHR Toolkit* that equips participants to integrate SRHR into community and policy dialogues, leading to increased knowledge uptake and local advocacy actions. Religious leaders trained through this programme have begun incorporating SRHR messaging into institutional programmes, broadening acceptance and building bridges with duty bearers.⁵⁹ Media actors are also increasingly recognised as key intermediaries; a report by the Kenya Human Rights Commission showed how media narratives on SRHR when shaped by informed ethical reporting influence public opinion and help hold policymakers accountable.⁶⁰

In Uganda and Tanzania, faith and traditional leaders are gaining traction as SRHR advocates through structured dialogue and capacity-building initiatives. The SRHR Alliance Uganda's Interfaith Dialogues bring together religious and community leaders to align traditional and religious values with progressive SRHR goals, creating a platform for engagement with policymakers on issues affecting adolescents and communities.

In Tanzania, initiatives like UNESCO's adolescent SRHR toolkit for religious leaders help equip clerics with resources to engage their congregations and interpret SRHR within cultural contexts, enabling them to act as informed advocates and community influencers. Networks such as MenEngage Tanzania, HEAL International and TIBA demonstrate how engaging non-traditional actors through community dialogues can build grassroots support for SRHR and create entry points for dialogue with local duty bearers.⁶¹ These evolving capacities enhance non-traditional allies' potential to participate more effectively in policy advocacy and accountability mechanisms, including advancing adoption and implementation of SRHR-related UPR recommendations

Level of participation of non-traditional SRHR allies with duty bearers in the review of SRHR-related laws, policies, and budgets.

Kenya: The non-traditional SRHR allies such as faith leaders and traditional community leaders are increasingly engaging with duty bearers in law, policy, and budget discussions, helping to shape a more inclusive health agenda. For example, inter-faith groups representing Catholic, Protestant, and other religious bodies convened with government and UN partners in May 2024 to develop an operational framework addressing HIV, gender-based violence, and adolescent pregnancy directly contributing to policy thinking on adolescent SRHR issues.⁶²

Faith actors are also participating in forums like the *7th Scientific Conference on Adolescent, Youth, and Sexual Reproductive Health Rights in Africa*, where religious leaders (e.g., Rev. Jacob Musili and Sheikh Ibrahim Lithome) engage policymakers and civil society to highlight legislative and budget gaps hindering youth SRHR access.⁶³ At the

⁵⁹ [Youth Changers Kenya](#)

⁶⁰ [Kenya Human Rights Commission \(KHRC\)](#)

⁶¹ [MenEngage Africa Alliance](#)

⁶² [UNESCO 2025](#)

⁶³ [Faith to Actions Network 2024](#)

community level, groups like the *Kenya SRHR Alliance* support youth champions in engaging elders and opinion leaders on issues such as early marriage and FGM, bringing these perspectives into broader SRHR policy advocacy.

However, the findings also noted that sociocultural and religious norms frequently stigmatize key aspects of SRHR, discouraging many elders and traditional leaders from openly supporting or participating in policy reviews or budget advocacy, and sometimes swaying lawmakers to downplay or avoid contentious SRHR issues. These challenges mean that, even when invited to the table, non-traditional allies may struggle to engage fully or influence outcomes in SRHR law and policy processes.

Uganda: The study findings established that SRHR Alliance Uganda has convened *annual interfaith dialogue forums* bringing together religious leaders from multiple denominations and cultural leaders to promote positive social norms, address adolescent SRHR challenges, and align traditional values with progressive policy goals. These platforms help integrate community and faith perspectives into broader SRHR discussions and raise awareness among policymakers and local leaders.

The findings also revealed ADRA Uganda engaged inter-faith leadership in a *national dialogue conference on adolescent health* that included faith actors in exploring faith-informed solutions to SRHR issues, fostering collaboration with health professionals and policymakers.⁶⁴ Initiatives like *intergenerational and religious dialogues* by Reach A Hand Uganda have brought elders, religious leaders, and political leaders together to discuss SRHR challenges such as teenage pregnancy and HIV, bridging generational and cultural divides to inform community-level advocacy and policy engagement.

However, the findings also noted that majority of the non-traditional allies in Uganda are rooted in norms that stigmatize contraception, abortion, and adolescent sexuality, which can make open engagement in policy debates difficult or unpopular within their communities, even when frameworks like the National Sexual and Reproductive Health and Rights Policy exist. Surveys and studies show that religious and traditional stakeholders are frequently excluded or hesitant to engage deeply in SRHR programming because of prevailing negative attitudes toward sexuality education and reproductive services, and community voices tied to these leaders are often underrepresented in formal policy influence.

Tanzania: Findings from the study reported that faith leaders trained under the *Chaguo Langu Haki Yangu* programme implemented by UNFPA and Tanzania Interfaith Partnership have committed to collaborating with government social welfare departments, police gender desks, and local paralegals to address harmful practices such as child marriage, gender-based violence (GBV), and female genital mutilation, thereby influencing community behavior and signaling support for related policy priorities.⁶⁵

⁶⁴ [ADRA Uganda 2024](#)

⁶⁵ [UNFPA Tanzania 2025](#)

The findings also revealed that religious leaders across multiple regions have been equipped with advocacy tools and messaging to actively raise awareness about violence prevention and positive gender norms in partnership with UNICEF and UNFPA platforms, reaching hundreds of thousands of community members.⁶⁶ Initiatives involving local pastors and theologians in family planning discussions have also linked religious voices to health system messaging on contraception uptake, illustrating how community leaders can indirectly shape policy discourse and acceptance of SRHR services.

Indicator 5: Inclusion of CSO recommendations in policy and budgetary decisions/proposals/reviews

SRHR-related recommendations identified by SRHR social movement to align them with responsible government stakeholders, timelines, and monitoring indicators.

Uganda: The findings established that CEHURD are pushing for increased budgetary allocation for family planning and adolescent health, strengthened implementation of the National SRHR Policy and accountability mechanisms to reduce unmet need for contraception. Other key recommendations mentioned by respondents that were identified by SRHR social movements focused on pressing issues like combatting teenage pregnancy, LGBTQI rights and compulsory education on sexual and reproductive health for young people. The findings also highlighted that adoption by government tends to focus on recommendations that are actionable and contextually relevant, reflecting efforts to harmonize regional guidance with national priorities.

However, the findings noted that despite adoption, ministries and institutions face significant challenges in implementing SRHR recommendations. Key barriers include limited resources, sociocultural perceptions, and resistance to sensitive interventions, particularly for marginalized groups. The complexity of tailoring interventions for minorities, including persons with disabilities, and navigating suspicion around SRHR agendas creates a difficult environment for effective implementation. These challenges highlight the need for additional advocacy, capacity building, and resource allocation to ensure that SRHR recommendations are fully realized in practice.

“One of the biggest challenges is the lack of resources to implement SRHR proposed recommendations adopted in policy. Number two, how people perceive SRH issues is different. It comes with a lot of suspicion. People think you are working for some foreign interests and trying to have their agenda go through.”

KII with Public-sector respondent, Uganda

Kenya: The findings established that advocates like SRHR Alliance and GEM Trust are calling for removal of age and consent barriers to adolescent sexual and reproductive health services and for the enactment of a comprehensive Sexual and Reproductive Health Act to harmonize SRHR protections. The study findings also revealed that key SRHR recommendations identified and proposed by CSOs and SRHR actors in Kenya targeted key health and rights issues, including maternal health services, HIV response, teenage pregnancy, gender-based violence (GBV), and access to universal health coverage (UHC).

⁶⁶ [UNICEF Tanzania 2024](#)

Respondents emphasized the importance of data-driven recommendations to improve health outcomes. By highlighting gaps in service provision and evidence-based policy needs, SRHR actors ensured that UPR submissions reflected both systemic and population-level challenges affecting marginalized groups, especially young people.

“The recommendations that we were developing were pushing for improvement in the maternal health services. We were also looking at having the effort towards UHC, universal health coverage, and HIV response strengthened. Our recommendations also focus on shedding light and showing the government how data is critical in pushing the overall health outcomes of the country, because if you do not know what you are ailing from, you will not know what medicine to take. We could be busy treating symptoms instead of sorting out the root cause, which will make sure that we are all okay.”

KII with Representative from GEM Trust

However, the findings also revealed that sociopolitical and cultural factors influence the selective adoption of SRHR recommendations. The respondents engaged in the study indicated that opposition from political, religious, and cultural groups may limit full acceptance of certain recommendations, particularly those related to sensitive areas such as adolescent sexual and reproductive health. This finding highlights the need for tailored advocacy strategies that consider local socio-cultural contexts and engage key stakeholders to facilitate broader uptake and implementation of SRHR measures.

Tanzania: The study findings reported that CWHRD advocate for implementation of youth-friendly service standards, improved access to post-abortion care, and legal clarity to protect women’s reproductive autonomy. These recommendations reflect mobilisation around persistent legal, budgetary and access challenges that organisations seek to address through national advocacy and engagement with duty bearers. Other key recommendations identified and advocated for by SRHR social movements in Tanzania focused on enabling adolescent girls to continue schooling after pregnancy, ensuring access to menstrual hygiene products, and improving health facility infrastructure. Discussions with SRHR actors also highlighted that CSOs and movements have been pursuing legal reform and strategic litigation, including initiating a High Court case to clarify the law and domesticate the Maputo Protocol in Tanzania.

However, the findings also revealed that even when recommendations are formally accepted, challenges remain in implementation, awareness creation, and community engagement. While a few policies, such as the PNC policy, may reflect partial adoption, the general understanding of SRHR-related recommendations remains unclear, indicating gaps in dissemination and knowledge among civil society actors. Stakeholders noted that while government reports on SRHR commitments identified by SRHR movements exist, the practical uptake at local and organizational levels is weak. There is a significant gap in capacity building and information dissemination, with many actors, including government officials and community members, unaware of frameworks like UPR and ICPD, making monitoring and advocacy difficult.

Existence of a UPR SRHR Implementation Plan developed in Kenya to serve as a reference tool for coordinated CSO advocacy and government engagement on SRHR implementation.

Kenya: The desk review findings confirmed that Kenya has an official UPR implementation plan that incorporates human rights recommendations, including those relevant to health and by extension SRHR, but there is no *separate SRHR-specific UPR implementation plan* publicly available. The Government of Kenya developed a comprehensive UPR Implementation Plan for the *Third Cycle (2021–2025)* following its 2020 UPR review, which lays out accepted recommendations, responsible actors (including ministries and civil society), indicators, timelines, and actions to implement the recommendations. This plan covers all rights areas reviewed and SRHR issues would be embedded within it where relevant recommendations were accepted but it is a general UPR implementation framework rather than a stand-alone SRHR plan.⁶⁷

Kenya's national mechanism for implementation, reporting, and follow-up (NMIRF) also uses an implementation matrix to track progress against UPR recommendations, coordinating across ministries such as Health and Justice; this matrix structure is the operational basis for follow-up rather than a unique SRHR plan. Civil society groups, including the *Kenya SRHR Alliance* and members of the *UPR Kenya* stakeholders' coalition, have advocated for integrating SRHR priorities into UPR follow-up and monitoring frameworks, but there is no evidence of a government-endorsed standalone UPR SRHR Implementation Plan published to date.

Level of government stakeholders' awareness and engagement with SRHR UPR recommendations as evidenced by their participation in advocacy dialogues.

Across the three countries, the findings revealed that awareness of SRHR recommendations under the UPR mechanism is uneven among stakeholders, with some government officials closely involved in human rights processes being fully informed and engaged, while others are not aware at all. Respondents who are part of formal human rights committees or who participate in inter-ministerial coordination reported receiving recommendations regularly and being involved in responding to them through their respective ministries. This includes implementing actions such as promoting girls' enrollment in education, affirmative action measures, and expanding technical and vocational training opportunities.

However, gaps exist among youth officers and other stakeholders who are not directly involved in these processes, highlighting the need for broader dissemination and capacity-building to ensure all actors within SRHR movements are aware of the recommendations and their implications for advocacy and programming. Engagement with the UPR provides opportunities to strengthen accountability, align national policies with international

⁶⁷ [Office of the Attorney General Kenya 2025](#)

standards, and inform strategic advocacy interventions at both national and regional levels.

“I often receive information because I participate in a human-rights coordination structure. UPR recommendations are assigned to the relevant sectors, which then report on implementation; for example, education recommendations may require evidence on girls’ enrolment and related measures.” –

KII, CSO in Uganda

CSOs recommendations on SRHR included in Policy and budget proposals in Kenya indicating support from relevant stakeholders

The study findings confirmed that key proposals from coalitions such as the SRHR Alliance and partners like GEM Trust include calls for removal of legal and policy barriers to adolescent access to sexual and reproductive health services and the enactment of a comprehensive Sexual and Reproductive Health Act to harmonize protections across law and policy. CSOs have also pushed for increased budget allocations for family planning commodities and reproductive health supplies, resulting in budget lines for procurement of family planning commodities (e.g., KSh 1.0B in the 2024/25 MTEF) and integrated reproductive health programmes supported through GoK/UNFPA partnerships. CSOs recommendations have influenced the inclusion of maternal and newborn unit upgrades and strengthened financing for youth-friendly SRH services in national and county planning frameworks, demonstrating a shift toward more rights-based SRHR funding in government budgets.

Existence of UPR SRHR Implementation Plan developed in Uganda and Tanzania to align them with responsible government stakeholders, timelines, and monitoring indicators.

Uganda: The desk review findings confirmed that there is no publicly available Uganda-specific SRHR-only UPR implementation plan that focuses solely on sexual and reproductive health and rights (SRHR). Uganda participates in the UPR mechanism and has accepted recommendations related to health and human rights, with some calls in earlier cycles for national action plans to follow up on UPR recommendations but this pertains to general UPR follow-up rather than a dedicated SRHR implementation plan.⁶⁸ The findings further noted that what *does* exist are general national UPR implementation and follow-up processes that civil society participates in. For example, coalitions convene mid-term UPR review conferences that assess implementation of accepted recommendations broadly, including those relevant to SRHR.

Tanzania: Similarly, the findings noted that Tanzania has not published a standalone UPR SRHR implementation plan that specifically organizes SRHR-related recommendations into an operational plan. The government participates in UPR cycles

⁶⁸ [Center for Reproductive Rights](#)

and accepts recommendations on a range of human rights issues, and civil society has conducted mid-term assessments on UPR implementation noting progress and gaps across accepted recommendations. Tanzania's human rights action frameworks such as gender equality and violence action plans may reference implementation of UPR and other human rights mechanisms collectively, but there is no official SRHR-focused UPR implementation plan publicly published that systematically tracks SRHR recommendations by itself.

Level of implementation of SRHR 4th cycle recommendations established in Kenya, resulting in review of the implementation plan.

The findings established that in Kenya, the State accepted and committed to implementing SRHR recommendations at its 4th cycle UPR, with public commitments to expand family planning and strengthen maternal health services, improve access to emergency obstetric care and enhance public awareness of sexual and reproductive health, including youth-specific services at the Human Rights Council adoption session. However, the level of implementation at baseline could not be clearly substantiated.

3.3 Existing Structures and Formation of SRHR Social Movements

SRHR social movements are grassroots and global efforts advocating for the right of individuals to make their own decisions about their bodies and their sexual and reproductive lives. These movements have developed over time as people-centered networks linking community activism with regional and global advocacy. These movements have emerged within politically sensitive environments, where debates around sexuality and women's rights are often polarized. Their structures, therefore, tend to be flexible and responsive, adapting quickly to political shifts and funding realities.⁶⁹

3.3.1 Structure of SRHR Social Movements in Kenya, Uganda and Tanzania

- a. Grassroots and Community-led Actors:** At the heart of these movements are community-based initiatives mostly women-led, youth-led, and sometimes Indigenous or faith-affiliated. They are often the first responders to SRHR challenges such as early marriage, female genital mutilation, and teenage pregnancy. Their structures are deliberately informal to encourage participation, mutual trust, and accountability to local communities.⁷⁰
- b. CSOs and National NGOs:** CSOs and NGOs provide the organizational backbone for SRHR work. They help connect grassroots advocacy to national and regional platforms, translate lived experiences into policy recommendations, and offer technical and legal expertise. In Kenya, Uganda, and Tanzania, groups like GEM Trust, CWHRD, CEHURD, KELIN, and TGNP have been instrumental in pushing for policy reforms and accountability in reproductive health services.

⁶⁹ [The State of African Women Report \(2018\)](#)

⁷⁰ [UNFPA Stories of Change \(2015\)](#)

- c. Regional and International Coalitions:** To amplify their influence, national actors often join regional and international coalitions. Networks such as the Eastern Africa National Networks of AIDS Service Organisations (EANNASO) and the Generation Equality Forum's SRHR Action Coalition bring together partners from multiple countries to coordinate advocacy and share strategies. This collaboration gives local voices access to continental and global policy spaces, where they can influence frameworks such as the Maputo Protocol and the ICPD Programme of Action.⁷¹ Frequent stock-outs of contraceptives and essential SRHR commodities, combined with inconsistent application of clinical and referral guidelines, further undermine service quality.
- d. Movement Champions and Allies:** The strength of these movements also lies in the diverse individuals who champion SRHR causes such as lawyers, journalists, health workers, religious leaders, and progressive policymakers. Their influence helps normalize the conversation around sexual rights, broadening it from a moral debate to an issue of public health and governance.⁷²

3.3.2 Formation of Social Movements in Kenya, Uganda and Tanzania

SRHR movements in East Africa do not emerge in isolation; they often form in response to inequality, discrimination, and restrictive laws that silence discussions about reproductive health. When civic space contracts or when backlash against women's and LGBTQIA+ rights intensifies, movements reorganize themselves through informal networks, social media, and cross-border partnerships. Their evolution, therefore, reflects a balance between resistance and adaptation.⁷³

These movements form through Rights-based legal advocacy which is visible through the UPR SRHR Thematic Group and legal networks in Kenya, CEHURD-supported coalitions in Uganda, and women-human-rights-defender and legal-reform coalitions in Tanzania. Youth organizing is channeled through NAYA Kenya and allied youth networks; Uganda Youth and Adolescents Health Forum (UYAHF), the African Network of Youth-led Organisations and Girls with Rights in Uganda; and Young and Alive Initiative and other youth-led formations in Tanzania.

Feminist and bodily-autonomy organizing includes GEM Trust-linked women's rights networks in Kenya, the Coalition to Stop Maternal Mortality Due to Unsafe Abortion (CSMMUA) in Uganda, and CWHRDs-linked women defenders in Tanzania. Other key examples of how social movements operate within the three countries include;

- a. Rights-Based and Justice-Oriented Foundations:** Most SRHR actors in the region are grounded in human rights and social justice principles. They draw legitimacy from national constitutions and international instruments such as CEDAW and the Maputo Protocol. The reproductive justice framework, which links reproductive autonomy to

⁷¹ [HEARD- SRHR in East and Southern Africa \(2023\)](#)

⁷² [The Kenya Legal & Ethical Issues Network on HIV and AIDS \[KELIN\] \(2023\)](#)

⁷³ [The EAC and Women and Girls' Rights \(2018\)](#)

broader economic and social rights, is increasingly shaping feminist and youth-led advocacy.⁷⁴

- b. Shifting Social Norms:** Changing harmful gender and social norms is central to how SRHR movements operate. Activists use dialogue, storytelling, community theatre, and media campaigns to challenge stigma and promote new narratives around sexuality, consent, and bodily integrity. This social transformation work is slow and long-term, but it lays the foundation for sustainable policy change.⁷⁵
- c. Legal and Policy Advocacy:** Policy and legal reforms remain key strategies. Movements engage in strategic litigation and push for implementation of progressive laws, while also monitoring budgets and public accountability. In the region, the *EAC's Sexual and Reproductive Health Bill of 2021* has become a rallying point for advocacy around regional commitments.⁷⁶
- d. Collaboration Across Generations and Sectors:** A defining feature of SRHR movements in East Africa is their intergenerational and multispectral approach. Established women's rights groups now mentor younger activists who bring digital skills and creative strategies to the table. Partnerships have also expanded to include education, climate justice, and disability rights groups, reflecting a more holistic understanding of sexual and reproductive justice.⁷⁷
- e. Creativity and Resistance:** In settings where open activism can attract hostility, creativity becomes a form of resistance. Art, music, and storytelling have become vital tools for expression and mobilisation. This creativity not only reaches new audiences but also helps sustain activists' morale and unity in difficult environments.⁷⁸

3.3.2. Types of SRHR Social Movements

- a. Feminist and Women's Rights Movements:** Feminist and women's rights movements are among the oldest and most influential SRHR actors in East Africa. Grounded in gender equality and social justice, they advocate for reproductive autonomy, access to family planning, safe abortion, and protection from gender-based violence. These movements use both community mobilisation and policy advocacy to push for laws that recognize women's reproductive rights as fundamental human rights.⁷⁹
- b. Youth-Led Networks:** Youth-led SRHR movements have grown rapidly over the past decade, bringing new energy, creativity, and digital innovation to advocacy work. Organisations such as the Network for Adolescent and Youth of Africa (NAYA) Kenya and Reach a Hand Uganda (RAHU) have been instrumental in promoting CSE and YFS. They represent a generational shift in SRHR organizing one that combines activism with peer education, social media campaigns, and direct policy engagement.⁸⁰
- c. HIV and AIDS Movements:** HIV movements are prominent in all three countries and often provide established service, accountability and key-population platforms that

⁷⁴ [KIT Regional Context Analysis of SRHR in Sub Saharan Africa \(2020\)](#)

⁷⁵ [2gether4SRHR \(2025\)](#)

⁷⁶ [EAC SRH Bill \(2021\)](#)

⁷⁷ [Women's Link Worldwide \(2025\)](#)

⁷⁸ [FEMNET \(2020\)](#)

⁷⁹ [FEMNET \(2020\)](#)

⁸⁰ [UNFPA Stories of Change \(2015\)](#)

overlap with wider SRHR advocacy. Examples in the evidence include Kenya Legal and Ethical Issues Network on HIV and AIDS (KELIN) and networks of people living with HIV in Kenya; AIDS Healthcare Foundation Uganda, Uganda Network of AIDS Service Organisations and HIV-linked RMNCAH coalitions in Uganda; and HIV-service and women's-rights networks in Tanzania.

- d. Legal Rights and Safe-Abortion Movements:** Legal and bodily-autonomy movements use litigation, legal literacy, policy reform and coalition advocacy to address unsafe abortion and maternal mortality. In Uganda, the Coalition to Stop Maternal Mortality Due to Unsafe Abortion is a multidisciplinary civil society group committed to the reduction of unsafe abortion in Uganda. Its secretariat is managed CEHURD. In Kenya, entities SRHR alliance, KELIN Kenya and FIDA Kenya drive strategic litigation, legal aid, and public health advocacy. In Tanzania, Networks like the Women's Global Network for Reproductive Rights Africa and local human rights groups organize public campaigns to decriminalize abortion.
- e. LGBTQIA+ and Key-Population Movements:** LGBTQIA+-led movements focus on combating stigma, discrimination, and criminalization that limit access to SRHR services for sexual and gender minorities. In Kenya, the National Gay and Lesbian Human Rights Commission (NGLHRC) has challenged discriminatory laws through the courts. In Uganda, Sexual Minorities Uganda (SMUG), before its deregistration pioneered community health education, while in Tanzania, CHESA has worked to integrate rights-based approaches into health service delivery. These groups play an essential role in advancing the principle that SRHR is universal and indivisible.⁸¹
- f. Disability-Inclusive SRHR Movements** - Disability-inclusive movements advocate for accessible reproductive health information, inclusive facilities, and trained service providers. In East Africa, networks such as the United Disabled Persons of Kenya (UDPK), the National Union of Women with Disabilities of Uganda (NUWODU), and SHIVYAWATA in Tanzania have integrated SRHR into their advocacy. They challenge both structural and attitudinal barriers that prevent persons with disabilities especially women and girls from exercising their sexual and reproductive rights.⁸²
- g. International and Regional Frameworks Guiding SRHR Movements** - The work of SRHR movements is grounded in international human rights frameworks that affirm reproductive rights as fundamental freedoms. The *International Conference on Population and Development (ICPD, 1994)* and its ICPD25 Nairobi Commitments (2019) provide global direction, linking reproductive rights to sustainable development. Regionally, the *Maputo Protocol (2003)*⁸³ and the *Maputo Plan of Action (2016–2030)*⁸⁴ guide African governments on policy, service provision, and accountability for SRHR. These instruments offer common language and legitimacy for advocacy efforts across Kenya, Uganda, and Tanzania.

3.3.3. SRHR Coalitions and Thematic Focus

⁸¹ [Human Rights Watch \(2020\)](#)

⁸² [UNFPA Young Persons with Disability \(2018\)](#)

⁸³ [AU Commission Protocol to the African Charter on Human and People's Rights on the Rights of Women in Africa \(2003\)](#)

⁸⁴ [AU Commission Maputo Plan of Action \(2016-2030\)](#)

Kenya: Kenya's SRHR movement ecosystem is anchored by a mix of national coalitions and community-based networks. NAYA Kenya coordinates more than 50 youth organisations advocating for comprehensive sexuality education and youth-friendly services. NAYA's 2023 annual report documents policy engagements in 16 counties and partnerships with the Ministry of Health's Division of Adolescent and School Health.⁸⁵ RHNK links service providers and advocates defending access to post-abortion care and family-planning commodities; it played a lead role in court petitions challenging county-level restrictions on reproductive health.⁸⁶

Feminist organisations such as FIDA-Kenya, CREAM, and Equality Now Africa jointly lead the KeepThePromise campaign, monitoring national commitments under the Nairobi Summit on ICPD25.⁸⁷ These groups also coordinate through the Kenya Sexual and Reproductive Health and Rights Alliance (KSRHRA), a broad umbrella supported by Amplify Change.

LGBTQIA+ advocacy is spearheaded by the National Gay and Lesbian Human Rights Commission (NGLHRC), GALCK+, and AFRA-Kenya, which focus on legal representation, strategic litigation, and health-rights monitoring. NGLHRC's litigation against forced anal testing and discrimination set a precedent for bodily-autonomy jurisprudence. Disability-inclusive advocacy is championed by the UPDK and the Association for the Physically Disabled of Kenya (APDK). Both partner with UNFPA to train health-care workers on inclusive SRHR communication and accessible clinic design.⁸⁸

Uganda: Organisations such as The Uganda Youth and Adolescent Health Forum (UYAHF) coordinate and represents multiple youth-led organisations, including regional and national youth networks; Reproductive Health Uganda participates in and supports coalitions; and CEHURD provides legal, policy and secretariat support to issue-based coalitions including CSMMUA. Feminist organisations including Uganda Women's Network (UWONET) and Women's Probono Initiative (WPI) advocate for gender-responsive health budgets and protection from gender-based violence, integrating SRHR into wider equality campaigns. The platforms listed above are illustrative rather than exhaustive and should be updated as the project's movement mapping expands.

LGBTQIA+ health and rights advocacy is carried out often under severe constraints by Sexual Minorities Uganda (SMUG), Human Rights Awareness and Promotion Forum (HRAFP), and allied collectives focusing on safe-space creation and HIV-SRHR integration. HRAFP's 2023 brief outlines the impact of criminalization on access to reproductive health services. Disability-inclusive actors, notably the National Union of Women with Disabilities of Uganda (NUWODU) and National Union of Disabled Persons of Uganda (NUDIPU), implement projects that train health providers and produce accessible SRHR materials for young persons with disabilities.⁸⁹

⁸⁵ [NAYA Kenya Strategic Plan \(2024-2028\)](#)

⁸⁶ [RHNK Annual Report \(2022\)](#)

⁸⁷ [FIDA Kenya Annual Report \(2023\)](#)

⁸⁸ [UNFPA State of World Population \(2023\)](#)

⁸⁹ [UNFPA Annual Report \(2023\)](#)

Tanzania: Tanzania's key SRHR coalitions include the Coalition for Women Human Rights Defenders (CWHRDs) and the Tanzania Gender Networking Programme (TGNP). Both coordinate feminist and community-based groups to advocate for equitable SRHR financing and the enforcement of reproductive-health policy commitments.⁹⁰

Youth-led initiatives such as Afya Plus and She Decides Tanzania mobilise secondary-school and out-of-school youth for reproductive-health awareness, mentorship, and advocacy with local councils.⁹¹ LGBTQIA+ organising remains sensitive, yet groups like CHESA, Community Health Education Services and Advocacy engage quietly through health-rights and HIV-integration programmes.

Disability-focused networks, primarily SHIVYAWATA, collaborate with government and UN agencies to adapt reproductive-health materials into sign language and Braille formats, ensuring persons with disabilities are represented in policy consultations.

3.3.5 Governance and Leadership Models

Kenya: Kenya's SRHR social movements display a combination of formal and semi-formal governance arrangements. Established organisations such as FIDA-Kenya, CREAM, and RHMK operate under registered NGO frameworks, each governed by boards of directors and guided by constitutions that define decision-making, accountability, and strategic oversight. Their annual reports show clear internal policies on leadership renewal, gender balance, and financial integrity.

Youth-led networks like NAYA Kenya and AMREF Youth in Action maintain participatory leadership models where young members elect representatives to national steering committees. This democratic approach ensures inclusivity and keeps agenda setting aligned with grassroots priorities.

Feminist alliances particularly those within the Kenya SRHR Alliance adopt collective governance through shared convening structures. Leadership rotates between member organisations, ensuring diverse representation across gender, regional, and thematic lines. Disability-inclusive groups such as UDPK and APDK have formal constitutions and national councils elected every three years, guaranteeing representation from county-level organisations of persons with disabilities.⁹²

Despite these strengths, fragmentation persists: different coalitions often operate parallel advocacy tracks, and the absence of a single national SRHR coordinating platform limits joint accountability and donor harmonization.⁹³

Uganda: Uganda's SRHR movement demonstrates a maturing governance culture shaped by strong legal and policy-advocacy organisations. CEHURD provides a model of

⁹⁰ [TGNP Strategic Plan \(2021 -2025\)](#)

⁹¹ [UNFPA Moving Forward Together \(2022\)](#)

⁹² [UNFPA Kenya Annual Report \(2023\)](#)

⁹³ [UPR Kenya 4th Cycle \(2024\)](#)

institutional governance, it is overseen by a board of trustees and guided by clear by-laws on strategic leadership, ethics, and risk management.⁹⁴

Youth coalitions like RAHU and P2P Uganda follow hybrid leadership structures combining executive directors with volunteer-based youth councils that steer programme design and communications. Feminist groups under UWONET and WPI use membership assemblies as their supreme governing bodies, electing boards that guide strategic direction, resource mobilisation, and policy engagement. Their governance manuals emphasise gender equity, transparency, and participatory decision-making.

Disability-inclusive actors such as National Union of Women with Disabilities of Uganda (NUWODU) and National Union of Disabled Persons of Uganda (NUDIPU) follow federated governance structures with district-level chapters feeding into national secretariats. These arrangements strengthen representation of rural women with disabilities in national SRHR dialogues.⁹⁵

However, CSOs face periodic regulatory scrutiny from the NGO Bureau, requiring frequent re-registration and limiting flexibility in leadership succession. Despite this, Ugandan SRHR organisations continue to practice collective accountability through coalitions like the SRHR Alliance Uganda, which convenes monthly coordination meetings of more than 25-member group.

Tanzania: In Tanzania, governance within SRHR movements blends formal NGO structures with loose advocacy coalitions. TGNP operates as a membership organisation with a general assembly that elects a board every four years, ensuring representation of grassroots women and youth groups.⁹⁶

The CWHRDs applies a rotational coordination model where different member organisations host the secretariat on a bi-annual basis to distribute leadership and reduce burnout. This approach has enhanced transparency and ownership across feminist networks.

Youth-led initiatives such as She Decides Tanzania and Afya Plus operate under simplified governance models registered as CBOs. They elect steering committees that include local government youth officers, creating a link between civic actors and state institutions.

Disability-rights networks like SHIVYAWATA maintain a national executive committee supported by thematic sub-committees on health, education, and livelihoods. Their SRHR working group includes representatives from six disability categories, reflecting inclusive leadership practice.

⁹⁴ [CEHURD Annual Report \(2022\)](#)

⁹⁵ [UNFPA Kenya Annual Report \(2023\)](#)

⁹⁶ [TGNP Strategic Plan \(2021 -2025\)](#)

Although restrictive NGO laws and administrative registration requirements sometimes slow leadership transitions, internal accountability mechanisms remain robust, and periodic coalition reflection meetings allow power-sharing and joint decision-making.⁹⁷

3.4 Organization and Advocacy Capacities

The desk review findings revealed that SRHR organisations across Kenya, Uganda, and Tanzania demonstrate varying levels of technical, institutional, and strategic capacity. CSOs and movements possess strong foundations in policy research, strategic communication, and advocacy engagement, particularly feminist organisations, youth-led networks, and disability-rights actors who routinely produce policy briefs, UPR submissions, and evidence-based advocacy materials. Technical capacity is strongest among well-established CSOs, while community-based groups continue to face gaps in research skills, documentation, and digital advocacy tools.⁹⁸

Strategic positioning and partnerships are strongest among organisations embedded in national coalitions or linked to continental networks such as FEMNET, AFYA+, and regional feminist alliances. These networks enhance visibility, create access to regional advocacy spaces, and enable coordinated responses to shrinking civic space. However, movement fragmentation and inconsistent participation continue to limit collective influence, especially in Tanzania where restrictive NGO regulations weaken advocacy coalitions.

Overall, strengths include established advocacy experience, legal expertise, youth mobilisation capacity, gender-responsive policy engagement, and strong regional linkages. Key challenges include limited financing, inadequate MEL systems for smaller actors, shrinking civic space, weak coordination tools, and reduced participation of disability and LGBTQIA+ organisations in decision-making spaces. Capacity development needs therefore include strengthening research and documentation skills, building sustainable MEL systems, improving coalition coordination, investing in digital advocacy, and enhancing protection mechanisms for human-rights defenders.

3.4 Coordination Platforms and Mechanisms

3.4.1. Regional Coordination

Regional coordination platforms in East Africa offer strategic frameworks for SRHR integration, legal harmonization, and youth engagement. Initiatives like 2gether4SRHR, convened by UN agencies, aim to align SRHR with development and humanitarian agendas across the region. Legal actors such as KELIN have facilitated cross-border dialogues with EALA, while youth-focused platforms like the East African Youth Network seek to amplify youth voices in governance and policy. However, these mechanisms often lack deep civil society ownership, and youth-led or feminist movements struggle to access and influence

⁹⁷ [Tanzania Human Rights Report \(2024\)](#)

⁹⁸ [CIVICUS 2023](#)

regional decision-making spaces. The challenge lies not in the absence of coordination structures, but in their accessibility, sustainability, and inclusiveness.

- a. **2gether 4 SRHR initiative**, convened by UNFPA, UNICEF, WHO, and UNAIDS, aims to embed SRHR into development and humanitarian programming across Eastern and Southern Africa. It supports regional scorecards, joint programming, and technical assistance, but remains largely donor-driven and institutionally led.⁹⁹
- b. **East African Centre for Human Rights (EACHRights)** plays a bridging role, convening across education, economic, social, and cultural (ECOSOC) rights, and youth participation. It is one of the few actors that explicitly links SRHR with education and human rights, offering a more integrated advocacy model that resonates with both policy and community actors.¹⁰⁰
- c. **Kenya Legal and Ethical Issues Network on HIV and AIDS (KELIN)**, though based in Kenya, plays a regional convening role through its legal advocacy work. KELIN has hosted cross-border dialogues with the East African Legislative Assembly (EALA) and civil society organizations to harmonize legal frameworks on abortion, CSE, and health rights. These efforts are impactful but episodic, often tied to donor timelines.¹⁰¹
- d. **East African Youth Network**, supported by the EAC, offers a platform for youth engagement across governance, SRHR, and education. However, its operational capacity is limited, and youth-led movements struggle to access regional policy spaces or influence agenda-setting.¹⁰²

3.4.2. Country Level Coordination

Kenya: Kenya's SRHR coordination ecosystem is marked by a blend of youth-led coalitions, legal reform platforms, and rights-based conveners. The country benefits from relatively well-established networks that engage across policy, service delivery, and civic education. However, despite this structural strength, coordination remains siloed, with limited integration between SRHR, education, and human rights actors. The presence of both statutory and civil society institutions offers opportunities for synergy but also reveals gaps in inclusion and cross-sectoral strategy. Below is a list of platforms coordination SRHR related matters in Kenya according to the study findings;

- a. **SRHR Alliance Kenya** is a dynamic youth-led movement that blends service delivery, policy advocacy, and community mobilization. With members like Africa Alive and AMREF Health Africa, it has positioned itself as a credible voice in national SRHR debates, especially around CSE and youth access to services. The Alliance engages county governments and national ministries, often leading youth-centered campaigns and policy dialogues.¹⁰³

⁹⁹ [UNFPA ESARO - 2gether 4 SRHR](#)

¹⁰⁰ [EACHRights - Strategic Plan](#)

¹⁰¹ [KELIN - EALA Convening](#)

¹⁰² [EAC Youth Engagement Strategy](#)

¹⁰³ [SRHR Alliance Kenya - About Us](#)

- b. Kenya Legal and Ethical Issues Network on HIV and AIDS (KELIN)** operates at the legal frontier, convening actors around constitutional health rights and hosting regional dialogues with bodies like the East African Legislative Assembly (EALA). KELIN's work includes strategic litigation, legal harmonization, and policy reform, though its framing can feel distant from youth-led or feminist organizing.¹⁰⁴
- c. Kenya Human Rights Commission (KHRC)** engages in legal reform, civic education, and youth rights, often intersecting with SRHR and gender justice. KHRC's long-standing credibility gives it access to policymaking spaces, though its engagement with SRHR is often framed through broader human rights lenses, which can dilute focus on youth-specific or gender-transformative agendas.¹⁰⁵
- d. Kenya National Commission on Human Rights (KNCHR)**, as a statutory body, monitors and reports on human rights violations, including those related to SRHR, GBV, and youth rights. While influential, its engagement with youth-led or feminist movements is limited, and its framing often reflects broader mandates rather than SRHR-specific priorities.¹⁰⁶

Uganda: Uganda's SRHR movement is energized by bold, youth-led platforms that have carved out space in national and regional advocacy. These actors are highly visible in digital and community spaces, and increasingly influential in policy dialogues. Yet coordination across legal, service delivery, and education sectors remains fragmented. While legal advocacy and donor-aligned service providers are active, they often operate in parallel to youth and feminist movements, limiting the potential for unified, cross-sectoral impact. Below is a list of platforms coordination SRHR related matters in Uganda according to the study findings;

- a. SRHR Alliance Uganda:** A consortium of youth-focused organizations promoting inclusive SRHR services and policy reform. The Alliance has been active in pushing for youth-friendly health policies and engaging in national dialogues on reproductive rights, though its influence is often constrained by political sensitivities around abortion and sexuality education.¹⁰⁷
- b. Uganda Youth and Adolescents Health Forum (UYAHF):** A youth-led platform integrating SRHR, gender, and education advocacy. UYAHF is known for its digital activism, community-based programming, and national-level engagement. It has emerged as a key voice in policy debates on CSE and YFS, often using social media and grassroots mobilization to amplify its reach.¹⁰⁸
- c. Center for Health, Human Rights and Development (CEHURD):** CEHURD combines strategic litigation, legal research, policy engagement and movement coordination. It serves as convener/secretariat of the Coalition to Stop Maternal Mortality Due to

¹⁰⁴ [KELIN - SRHR Curriculum](#)

¹⁰⁵ [Kenya Human Rights Commission - Programme Areas](#)

¹⁰⁶ [KNCHR - Reproductive Health Rights Report](#)

¹⁰⁷ [SRHR Alliance Uganda - Who we are](#)

¹⁰⁸ [UYAHF - Programs and Advocacy](#)

Unsafe Abortion (CSMMUA), supporting membership coordination, quarterly meetings, thematic working areas, collective advocacy and engagement with government and parliamentary actors.

- d. Reproductive Health Uganda (RHU):** Member of IPPF, active in service-delivery and policy engagement. RHU has strong ties to government and international donors, which gives it policy leverage but also positions it more as a service provider than a movement convener.¹⁰⁹

Tanzania: Tanzania's coordination platforms are rooted in a strong feminist tradition, with organizations like the TGNP leading structural advocacy on gender and economic justice. Service delivery actors such as Chama cha Uzazi na Malezi Bora Tanzania (UMATI), the national affiliate of the International Planned Parenthood Federation (IPPF), complement this with deep community engagement. However, coordination is largely donor-driven and episodic, with limited youth leadership and weak regional linkages. The divide between adult-led feminist platforms and emerging youth movements presents both a challenge and an opportunity for more inclusive, movement-led coordination. Below is a list of platforms coordination SRHR related matters in Tanzania according to the study findings;

- a. Tanzania Gender Networking Programme (TGNP)** is a movement-builder that links SRHR with economic justice and gender equality. It's known for its intersectional framing and policy advocacy, though it remains adult-led and less youth-centered.¹¹⁰
- b. Chama cha Uzazi na Malezi Bora Tanzania (UMATI)** IPPF affiliate, focuses on youth-friendly SRHR services, sexuality education, and community outreach. With over 60 years of experience, it has strong roots in health service delivery and peer education, but its policy influence is limited compared to actors like TGNP or CEHURD.¹¹¹
- c. Donor-led mechanisms like the United Nations Population Fund (UNFPA) Tanzania Roundtable** convene actors across SRHR, education, and nutrition. These spaces are strategic but episodic, often driven by external funding cycles and lacking mechanisms for sustained civil society leadership.¹¹²
- d. The GRREAT Initiative** (Girls' Reproductive Health, Rights, Empowerment, and Education through Adolescents and Technology), led by UNICEF and UNFPA, integrates SRHR, nutrition, and education for adolescent girls. Operating in Mbeya, Songwe, and Zanzibar, it aims to reduce barriers to adolescent-friendly services and improve cross-sectoral outcomes. While promising in design, it remains heavily reliant on external funding and top-down implementation.¹¹³

¹⁰⁹ [Reproductive Health Uganda - About](#)

¹¹⁰ [TGNP - About Us](#)

¹¹¹ [UMATI -Services and Programs](#)

¹¹² [UNFPA Tanzania - Partner Roundtable](#)

¹¹³ [UNICEF Tanzania - GRREAT Program](#)

3.5 Cross Cutting Challenges and Opportunities

3.5.5 Challenges

Kenya:

- a. Fragmented coordination across SRHR actors:** Despite the presence of strong coalitions such as the *UPR SRHR Thematic Group*, *RHNK*, and the *SRHR Alliance Kenya*, evidence shows that coordination remains largely issue-specific. Movements often work in parallel, feminist networks on GBV, youth networks on adolescent health, LGBTQIA+ groups on anti-discrimination, and legal organisations on litigation, without consistent cross-movement alignment. This fragmentation reduces opportunities for unified agenda-setting and weakens the sector's collective influence on policy processes.
- b. Limited integration of SRHR issues within mainstream government coordination mechanisms:** Findings show that while TWGs and sector working groups exist, SRHR participation varies widely. Several health-sector mechanisms function irregularly or lack consistent representation from key SRHR constituencies such as youth, LGBTQIA+, and disability groups. As a result, policy debates, guidelines, and planning processes often exclude the perspectives of those most affected by SRHR barriers. Moreover, KIIs highlighted that access to these mechanisms is uneven and often dependent on informal relationships or project-based engagement.
- c. Weak follow-up on UPR-related SRHR recommendations:** Although Kenya regularly receives SRHR-related recommendations through the UPR process, evidence shows limited translation of these recommendations into structured government action, planning, or budget commitments. There is no clearly operationalised national implementation plan for SRHR-related UPR actions, resulting in inconsistent tracking, inadequate reporting, and minimal accountability for progress.

Uganda:

- a. Inadequate financing and resource support:** Resource mobilisation remains weak due to heavy reliance on external donors, declining flexible funding for rights-based SRHR work, and limited domestic financing, which undermines long-term advocacy and monitoring of UPR commitments.
- b. Restrictive Political Environment:** Policy advocacy and lobbying are constrained by a restrictive legal and political environment, including laws and policies that limit civic space and criminalize aspects of sexuality, making government actors reluctant to adopt or implement SRHR-related UPR recommendations.
- c. Limited Coordination:** Coordination within the movement is also a challenge, with fragmented coalitions, uneven inclusion of grassroots and key population groups, and limited platforms for sustained joint action, reducing collective influence on national policy and UPR follow-up processes.

Tanzania:

- a. Policy advocacy constraints:** Conservative social norms and political sensitivities around SRHR issues particularly adolescent sexuality, family planning, and abortion

limit open debate and make policymakers hesitant to advance rights-based policies. This environment often leads to self-censorship among advocates and reduced willingness by duty bearers to engage on contentious SRHR topics, weakening advocacy impact.

- b. Limited uptake of UPR recommendations:** Despite civil society participation in the UPR process, government receptiveness to SRHR-related recommendations remains uneven, with many proposals on rights-based reproductive health receiving limited political traction. Advocacy efforts are further constrained by weak accountability mechanisms and insufficient political will to translate accepted recommendations into legal and budgetary action.
- c. Coordination of SRHR service delivery:** At the implementation level, fragmented coordination between government agencies, local authorities, and civil society partners leads to inconsistent SRHR service coverage. Poor integration of actors across regions, gaps in monitoring and data sharing, and insufficient alignment of budgets with strategic SRHR plans make it difficult to scale up youth-friendly services and ensure continuity of care particularly in rural and underserved communities.

3.5.6 Entry Points and Opportunities

Kenya:

- a. Leveraging established coordination platforms for unified SRHR advocacy:** Evidence suggests that existing structures such as the *UPR SRHR Thematic Group*, the *Kenya Stakeholders Coalition facilitated by EACHRights*, *RHNK*, and the *SRHR Alliance Kenya* offer immediate entry points for harmonising SRHR messaging, aligning evidence, and driving coordinated advocacy on UPR recommendations. Strengthening these platforms can help consolidate fragmented efforts into a more unified national agenda as these platforms already convene diverse actors and provide structured spaces for joint advocacy, knowledge sharing and engagement with duty bearers during national reporting cycles and policy windows.
- b. Using the UPR process as a unifying accountability mechanism:** Findings from the desk review and KIIs show that the *UPR cycle provides a strategic entry point for aligning SRHR advocacy across thematic areas*. The preparation of joint submissions, mid-cycle tracking, and follow-up dialogues create opportunities to consolidate evidence from GBV, ASRH, abortion, LGBTQIA+, and disability rights actors into a shared accountability agenda.
- c. Strengthening participation in government-led TWGs at county and national levels:** Existing *RMNCAH TWGs*, *GBV coordination committees*, *Ministry of Health multistakeholder platforms* and *adolescent health platforms* present actionable entry points for influencing government priorities and ensuring that SRHR-related UPR recommendations are reflected in planning, budgeting, and policy review cycles. Findings indicate that enhancing representation, especially for youth, disability, and LGBTQIA+ actors, would improve inclusion and ensure intersectional SRHR inputs. Additionally, targeted engagement, particularly during planning, budgeting and guideline review phases, can strengthen SRHR integration within health-sector priorities at both national and county levels.

Uganda:

- a. **Leveraging multi-stakeholder platforms and coalitions:** Existing networks such as the SRHR Alliance Uganda and the National Coalition of Human Rights Defenders (NCHRD-Uganda) provide platforms for coordinated advocacy on policy, budget, and accountability issues. These coalitions allow member organisations (e.g., *Reproductive Health Uganda*, *HEPS-Uganda*, and *Youth Coalition for Sexual and Reproductive Health*) to pool resources, share evidence, and engage duty bearers with unified SRHR agendas, enhancing visibility and policy impact.
- b. **Engaging in formal review and accountability mechanisms:** Civil society can deepen participation in national and international review cycles, including UPR mid-term reviews and the *Uganda Human Rights Commission's* platforms, to hold the government accountable for accepted SRHR-related UPR recommendations. For instance, the NCHRD-Uganda's UPR mid-term conference brought CSOs and government actors together to assess progress and plan follow-up action, creating space for structured dialogue and joint advocacy.
- c. **Strengthening youth and community-led advocacy:** Youth networks such as *Reach A Hand Uganda* and *Youth Coalition for Sexual and Reproductive Health* are well-positioned to drive local demand for SRHR services, influence local governments, and build grassroots support. Targeted capacity building for youth advocates enhances community engagement on critical issues like comprehensive sexuality education and access to services

Tanzania:

- a. **Strengthening partnerships and coordination:** The findings highlighted that SRHR advocates and CSOs can participate in periodic reviews and updates of key health policies (e.g., adolescent health, maternal and newborn care) to include stronger SRHR provisions, ensuring civil society voices contribute evidence and recommendations. SRHR organizations and implementing partners can also link community evidence and citizen feedback into broader advocacy campaigns, reinforcing grassroots legitimacy and pressure, participate in the submission of alternative reports and joint civil society assessments to hold government accountable to SRHR-related recommendations.
- b. **Utilizing evidence and research for strategic influence:** Organizations can collaborate with research partners to generate data on SRHR gaps, service delivery challenges, and social norms, strengthening policy briefs and advocacy messages. Evidence from pilot projects, community assessments, and program evaluations can support targeted lobbying for policy and budget reforms.
- c. **Strengthening budget advocacy through formal engagement with Parliament:** CSOs have opportunities to engage government stakeholders and parliamentary committees to advocate for increased domestic financing for SRHR. Platform Tanzania and community networks provide avenues to engage Ward Development Committees, Council Health Management Teams, and budget planning cycles at district and regional levels to push for SRHR allocations in local health plans.

- d. **Media and Public Communication Platforms:** Collaborating with journalists and media houses through strategic communications campaigns on SRHR topics helps amplify advocacy, shape public opinion, and create windows of opportunity for policymakers to respond. Media partnerships during awareness campaigns (e.g., on family planning or GBV) will help increase visibility and policy relevance.

4.1. Conclusions

The findings revealed that SRHR movements and CSOs in Kenya, Uganda, and Tanzania have made significant strides in strengthening SRHR policy frameworks, through coalition building, evidence-based advocacy, engagement with youth, and inclusion of non-traditional allies such as faith and community leaders. Their efforts have contributed to increased awareness, policy dialogue, and incremental adoption of rights-based SRHR measures, including integration of UPR recommendations into national priorities.

However, opportunities remain to enhance impact by deepening coordination across networks, expanding domestic resource mobilization, scaling youth-led and community-driven initiatives, and systematically tracking government implementation of SRHR commitments. Building on the strong foundation already laid, SRHR movements and CSOs can further consolidate gains, influence policy reforms, and ensure sustainable, inclusive access to SRHR services for all populations.

The study findings highlighted that SRHR movements and CSOs in Kenya, Uganda, and Tanzania have made notable progress in strengthening coordination, leveraging coalitions, multi-stakeholder platforms, and networks to unify advocacy efforts, share evidence, and engage policymakers more effectively. These collaborative approaches have enhanced visibility, amplified community voices, and created opportunities to influence SRHR laws, policies, and budgets, including engagement with UPR recommendations.

Despite these gains, there are opportunities to further improve coordination by fostering stronger regional and cross-sectoral partnerships, integrating grassroots and youth-led initiatives more systematically, harmonizing advocacy strategies, and establishing robust mechanisms for shared monitoring and accountability. Building on the foundation already established, enhanced coordination can increase collective impact, streamline resource use, and ensure more cohesive and sustainable progress in advancing SRHR across the region.

The findings confirmed that SRHR movements in Kenya, Uganda, and Tanzania have made important progress in advocating around UPR processes, engaging governments, submitting alternative reports, and participating in multi-stakeholder reviews to push for the adoption and implementation of SRHR-related recommendations. These efforts have elevated SRHR issues within national human rights dialogues, increased accountability, and strengthened civil society's voice in policy and budget discussions.

However, opportunities remain to enhance impact by expanding collaboration across networks, increasing the use of evidence-based advocacy, deepening engagement with non-traditional allies such as faith and community leaders, and establishing systematic follow-up mechanisms to track government action on accepted UPR recommendations. By building on these foundations, SRHR movements can more effectively influence policy reforms, ensure government accountability, and advance comprehensive, rights-based sexual and reproductive health and rights in the region.

SRHR movements and CSOs in Kenya, Uganda, and Tanzania have made significant strides in strengthening their internal capacities, including enhancing advocacy skills, evidence generation, strategic planning, and organizational governance. These efforts have enabled them to engage more effectively with policymakers, coordinate multi-stakeholder initiatives, and deliver community-driven SRHR programs. Opportunities exist to further build capacity by investing in specialized training on policy analysis, UPR processes, resource mobilization, digital advocacy, and monitoring and evaluation systems. Strengthening internal coordination, knowledge sharing across regional networks, and mentorship programs can also enhance resilience and sustainability.

4.2. Recommendations

#Strengthen CSO Advocacy on SRHR Policy Change and Implementation

The study findings propose that SRHR movements in Kenya, Uganda, and Tanzania should deepen coalition building and cross-sector partnerships, drawing on successful examples such as the Kenya SRHR Alliance's multi-stakeholder advocacy on family planning budgeting and the SRHR Alliance Uganda's National Leaders' Dialogue on adolescent health to shape high-level policy engagement. CSOs should invest in evidence-based advocacy, using research from institutions like *Reproductive Health Uganda* to inform policy briefs and budget submissions, and publish joint shadow UPR reports to hold governments accountable to SRHR-related recommendations. Strengthening engagement with non-traditional allies such as faith leaders involved in Tanzania's *Chaguo Langu Haki Yangu* interfaith platforms can broaden legitimacy and reach, while expanding capacity building in policy analysis, digital advocacy, and legislative engagement will enhance strategic influence in parliamentary and executive decision-making spaces.

#Increase CSO Participation In Policy And Budgetary Processes

The study findings highlight that SRHR movements in Kenya, Uganda, and Tanzania should actively engage in formal government planning and review platforms, leveraging successful models such as EACHRights and the Kenya SRHR Alliance's engagement with Parliamentary health committees on family planning budgets and the Civil Society Budget Advocacy Group (CSBAG) in Uganda, which provides structured spaces for evidence-based advocacy on health financing. In Tanzania, platforms like Platform Tanzania and district-level health planning committees offer entry points for CSOs to influence local and regional budget allocations for SRHR services. CSOs should also strengthen coalition-based advocacy, combining evidence generation, community feedback, and coordinated lobbying to enhance credibility and policy influence. The findings also note that integrating youth-led and grassroots voices, as demonstrated by Reach A Hand Uganda in budget consultations, ensures that resource allocation decisions are informed by the needs of key populations.

#Strengthen CSO Coordination on SRHR Advocacy and Policy Review

CSOs and movements in Kenya, Uganda, and Tanzania should build and leverage robust multi-stakeholder coalitions that bring together diverse actors for unified advocacy. For example, the Kenya SRHR Alliance has successfully coordinated member organizations to

engage policymakers on family planning and UPR-related recommendations, while SRHR Alliance Uganda convenes national dialogues that align civil society input on adolescent health policies. In Tanzania, networks such as Platform Tanzania and Women Fund Tanzania Trust-supported SRHR Coalitions provide structured spaces for joint strategy development and sharing of advocacy evidence. CSOs should institutionalize regular coordination meetings, joint policy briefs, and collaborative monitoring frameworks, ensuring consistent messaging and maximizing influence in policy review, budget discussions, and UPR follow-up processes. Strengthened coordination across these networks will enhance collective impact, reduce duplication, and improve government responsiveness to SRHR priorities.

#Enhance Monitoring of Implementation of SRHR-related UPR Recommendations

SRHR movements and CSOs in Kenya, Uganda, and Tanzania should establish systematic tracking and reporting mechanisms that capture government progress on accepted recommendations. This can include developing joint civil society shadow reports and conducting multi-stakeholder review forums. Strengthening data collection, periodic progress reviews, and coordinated advocacy strategies will enable CSOs to hold governments accountable, identify gaps, and inform targeted interventions, ensuring that SRHR-related UPR recommendations translate into tangible improvements in laws, policies, and service access.

CSOs in Kenya, Uganda, and Tanzania can also strengthen advocacy for the adoption and implementation of SRHR-related UPR recommendations by leveraging evidence-based research, coalition-building, and strategic engagement with duty bearers. In practice, CSOs can produce policy briefs and shadow reports highlighting gaps in SRHR service delivery, participate in national and county-level budget and policy consultations to influence resource allocation, and engage parliamentary committees, ministries of health and gender, and local governments to monitor UPR implementation

#Strengthen SRHR movements and CSOs Organizational Capacities

To strengthen SRHR movements and CSOs' organizational capacities, actors in Kenya, Uganda, and Tanzania should invest in targeted capacity-building initiatives that enhance skills in policy analysis, advocacy, research, monitoring and evaluation, and strategic communications. Enhancing internal systems, resource mobilization skills, and digital advocacy capacity will enable CSOs to operate more efficiently, sustain long-term programs, and respond adaptively to emerging SRHR challenges, ultimately increasing their collective impact on policy reform, service delivery, and rights-based advocacy.

#Promote Partnerships with Non-Traditional Allies and Actors

SRHR movements and CSOs in Kenya, Uganda, and Tanzania should strategically engage faith leaders, traditional leaders, media practitioners, activists, and entertainers to broaden support for rights-based sexual and reproductive health agendas. Collaborating with platforms such as interfaith and cultural dialogues and media engagement initiatives can help align community norms with SRHR policies, amplify advocacy messages, and increase public awareness. By building these cross-sectoral partnerships, CSOs can leverage trusted community voices, reach diverse audiences, challenge stigma, and

strengthen advocacy for policy change, budget allocations, and implementation of SRHR-related UPR recommendations, ensuring a more inclusive and sustainable approach to advancing sexual and reproductive health and rights.

ANNEXES

Annex I: Baseline Indicator Matrix

Indicator Milestone	Indicators	Baseline Findings
AmplifyChange grantee undertakes rigorous advocacy planning and/or regular advocacy plan reviews (including policy mapping, audience analysis, decision-making pathways in order to structure and target advocacy activities logically) for SRHR policy change.	SRHR social movements develop and adopt movement-strengthening and advocacy planning tools to engage with the UPR mechanism resulting in at least three coordinated national advocacy and movement-strengthening plans.	The findings highlighted that GEM Trust, CEHURD, CWHRD and allied SRHR CSOs use a range of advocacy tools and strategies to advance sexual and reproductive health and rights. CSOs participate in national coalitions such as the Universal Periodic Review (UPR) SRHR Thematic Group, SRHR Alliances that combines research, evidence-generation, and media advocacy to document barriers to SRHR and engage policymakers at county, national and regional levels. CSOs employ toolkits and structured advocacy frameworks to strengthen civil society capacity and shape policy debates: for example, regional SRHR advocacy toolkits provide guidance on policy analysis, stakeholder mapping, communications, and lobbying strategies that can be adapted for local use.
	The SRHR social movements adopt more adaptive and responsive strategies, resulting in more effective engagement with stakeholders on SRHR-related UPR recommendations.	The findings revealed that In Kenya, the GEM Trust conducts research and media advocacy on women's human rights and SRHR to inform policy and public discourse, combining evidence generation with targeted advocacy campaigns to strengthen women's autonomy in reproductive decision-making.

		In Uganda, CEHURD employs strategic litigation, advocacy and community empowerment to influence legal and policy frameworks that protect health rights, using documented cases and community mobilisation to hold duty bearers accountable for equitable SRHR access. In Tanzania, CWHRDs integrates SRHR into broader human rights advocacy by holding dialogues and capacity-building events, raising awareness about unsafe abortion and post-abortion care, and equipping women rights defenders to challenge stigma and legal barriers, while strengthening grassroots movements for legal transformation and feminist solidarity around sensitive SRHR issues
	Government stakeholders begin to take concrete action in support of SRHR-related UPR recommendations, indicating increased accountability and policy responsiveness to civil society advocacy.	The findings indicated that In Kenya, the State accepted and committed to implementing SRHR recommendations at its 4th cycle UPR, with public commitments to expand family planning and strengthen maternal health services, improve access to emergency obstetric care and enhance public awareness of sexual and reproductive health, including youth-specific services at the Human Rights Council adoption session. In Tanzania, the government has integrated SRHR priorities into national health strategies such as the Health Sector Strategic Plan V (2021–2025) and the One Plan III, and adopted National Standards and Implementation Guides for adolescent- and youth-friendly SRH services to

		improve quality and accessibility of SRH services for young people. In Uganda, the Ministry of Health has developed detailed National Policy Guidelines and Service Standards for Sexual and Reproductive Health and Rights to guide delivery of integrated, rights-based SRH services across service delivery points, and the government has invested in national strategies that focus on reducing unmet need for family planning, increasing contraceptive prevalence and improving adolescent and youth health outcomes
Increased CSO participation in policy and budgetary processes (e.g. Joint Annual Reviews, development of health sector strategic plans, national delegations on the country's SRHR commitments, etc.)	CSOs demonstrate improved advocacy skills, leading to increased engagement with duty bearers on SRHR-related UPR recommendations.	The findings highlighted that GEM Trust participates in national coalitions such as the SRHR Alliance, which convenes CSOs and regularly interfaces with ministries (Health, Gender, Education) and county governments to amplify community voices and advocate for rights-based SRHR policy implementation and accountability. CEHUR works on programmes such as the Stand Up for SRHR project, engaging government officials in district local governments and ministries during inception and implementation to align service delivery, policy reform and human rights obligations. CWHRD has organised SRHR dialogues and strategic planning events that bring civil society and stakeholders together to discuss unsafe

		abortion and post-abortion care, building capacity and recommending action to authorities to make SRHR information and services more accessible
	National-level SRHR advocacy becomes more strategic and unified, evidenced by the creation of three coordinated advocacy strategies addressing UPR recommendations.	The findings highlighted that in Kenya, the SRHR Alliance provides a structured national strategy for CSOs including GEM Trust to align advocacy on adolescent SRHR, maternal health, and legal reforms with UPR recommendations, using joint research, policy briefs, and stakeholder dialogues. In Uganda, coordinated efforts are evident through coalitions such as CEHURD's Stand Up for SRHR initiative and the Right Here Right Now (RHRN2) programme, which harmonise CSO actions on family planning, adolescent health, and equitable SRHR financing while engaging duty bearers. In Tanzania, the Coalition for Women Human Rights Defenders (CWHRDs) integrates SRHR into national advocacy strategies by combining community mobilisation, policy dialogue, and capacity-building of women's rights defenders to advance legal reforms and access to reproductive health services
	CSOs influence SRHR-related law, policy, and budget reforms in Kenya, Uganda, and Tanzania through sustained engagement with duty bearers informed by UPR recommendations.	GEM Trust through the SRHR Thematic Group has helped foreground recommendations that call for reviewing legal and policy barriers to SRHR access for adolescents, young people, persons with disabilities, people living with HIV and key populations, as part of Kenya's UPR

		stakeholder submissions and follow-up advocacy. CEHURD is pushing for increased government investment in SRHR financing, stronger family planning and adolescent health services and accountability for equitable reproductive health access in national budgets, emphasising expanded support for maternal health and reproductive care as part of its public budget advocacy and human rights work. CWHRD advances UPR-aligned SRHR goals by organising dialogues and capacity-building on unsafe abortion, post-abortion care and women's health rights, recommending measures to protect women and girls and strengthen SRHR service provision at community and policy levels
	Legislators, regulators, and other decision-makers demonstrate greater commitment to advancing SRHR by actively engaging in the review and reform of laws, policies, and budgets in line with UPR recommendations.	The findings noted that in Kenya, Parliament and relevant ministries have initiated reviews of the National Reproductive Health Policy (2022–2032) and are strengthening county-level budget allocations for maternal health, family planning, and adolescent SRHR services, aligning with UPR calls to remove legal and policy barriers to access.
	Legislators, regulators, and other decision-makers demonstrate increased engagement in reviewing and reforming SRHR laws, policies, and budgets in response to SRHR-related UPR recommendations.	In Uganda, lawmakers are revising national frameworks and investing in family planning and adolescent health programmes, guided by evidence from CSOs such as CEHURD, to ensure equitable and rights-based SRHR service provision.

		In Tanzania, policymakers are implementing reforms under the Health Sector Strategic Plan V and the One Plan III, including the adoption of standards for youth-friendly SRH services and budgeting for reproductive health commodities, reflecting UPR recommendations on improving access and legal protections for women and adolescents. These actions demonstrate growing political and fiscal commitment to translating UPR SRHR recommendations into concrete legal, policy, and resource decisions
AmplifyChange grantees strengthen their organizational capacity	own	The organizations demonstrate willingness to strengthen their own capacity to implement the recommendations from the Due Diligence report, effective SRHR advocacy and programming, following the adoption of the Due Diligence Report and the Organizational Capacity Assessment.
		The partners will have acted on the recommendations from the Due Diligence and the Organisational Capacity Assessment reports, thereby strengthening their capacities to successfully deliver the project.
		Organizational systems and practices in at least one core area are improved and applied in daily operations, as demonstrated through revised documents and team practice.
Partnerships established with non-traditional SRHR allies (including:	Non-traditional SRHR allies demonstrate increased advocacy capacity and engage	In Kenya, organisations such as Youth Changers Kenya have trained religious leaders, journalists,

religious leaders, faith movements, youth groups, social justice / budget tracking / research / media organisations that don't traditionally work in SRHR)	more effectively with duty bearers on SRHR-related UPR recommendations.	and policymakers using an <i>Intersectional SRHR Toolkit</i> that equips participants to integrate SRHR into community and policy dialogues, leading to increased knowledge uptake and local advocacy actions. Religious leaders trained through this programme have begun incorporating SRHR messaging into institutional programmes, broadening acceptance and building bridges with duty bearers In Uganda and Tanzania, faith and traditional leaders are gaining traction as SRHR advocates through structured dialogue and capacity-building initiatives. The SRHR Alliance Uganda's Interfaith Dialogues bring together religious and community leaders to align traditional and religious values with progressive SRHR goals, creating a platform for engagement with policymakers on issues affecting adolescents and communities. In Tanzania, initiatives like UNESCO's adolescent SRHR toolkit for religious leaders help equip clerics with resources to engage their congregations and interpret SRHR within cultural contexts, enabling them to act as informed advocates and community influencers.
	Increased participation of non-traditional SRHR allies with duty bearers in the review of SRHR-related laws, policies, and budgets	The findings established that in Kenya, faith leaders and county-level youth representatives engage with health and gender ministries through platforms like the SRHR Alliance to provide community perspectives during policy

		consultations and budget hearings on maternal health and adolescent SRHR. In Uganda, journalists and traditional leaders collaborate with policymakers and CSOs such as CEHURD during the Stand Up for SRHR initiatives to advocate for evidence-based reforms in family planning, adolescent health, and equitable SRHR financing. In Tanzania, youth networks and faith-based organizations participate in government-led consultations under the One Plan III and the Health Sector Strategic Plan V, contributing to discussions on adolescent-friendly services, reproductive health commodities, and safe abortion policies
Inclusion of CSO recommendations in policy and budgetary decisions/proposals/reviews	The members of the SRHR social movement review and/or identify the SRHR-related recommendations to align them with responsible government stakeholders, timelines, and monitoring indicators.	In Kenya, the findings established that advocates like SRHR Alliance and GEM Trust are calling for removal of age and consent barriers to adolescent sexual and reproductive health services and for the enactment of a comprehensive Sexual and Reproductive Health Act to harmonise SRHR protections. In Uganda, CSOs including CEHURD are pushing for increased budgetary allocation for family planning and adolescent health, strengthened implementation of the National SRHR Policy and accountability mechanisms to reduce unmet need for contraception.

		In Tanzania, CWHRD advocates for the implementation of youth-friendly service standards, improved access to post-abortion care, and legal clarity to protect women's reproductive autonomy. These recommendations reflect mobilisation around persistent legal, budgetary and access challenges that organisations seek to address through national advocacy and engagement with duty bearers
	The UPR SRHR Implementation Plan developed in Kenya to serve as a reference tool for coordinated CSO advocacy and government engagement on SRHR implementation.	The desk review findings confirmed that Kenya has an official UPR implementation plan that incorporates human rights recommendations, including those relevant to health and by extension SRHR, but there is no <i>separate SRHR-specific UPR implementation plan</i> publicly available. The Government of Kenya developed a comprehensive UPR Implementation Plan for the <i>Third Cycle (2021–2025)</i> following its 2020 UPR review, which lays out accepted recommendations, responsible actors (including ministries and civil society), indicators, timelines, and actions to implement the recommendations. This plan covers all rights areas reviewed and SRHR issues would be embedded within it where relevant recommendations were accepted but it is a general UPR implementation framework rather than a stand-alone SRHR plan
	Relevant government stakeholders demonstrate awareness of and engagement with SRHR UPR	Across the three countries, the findings revealed that awareness of SRHR recommendations under the UPR mechanism is uneven among stakeholders, with some government officials

	recommendations as evidenced by their participation in advocacy dialogues.	closely involved in human rights processes being fully informed and engaged, while others are not aware at all. Respondents who are part of formal human rights committees or who participate in inter-ministerial coordination reported receiving recommendations regularly and being involved in responding to them through their respective ministries. This includes implementing actions such as promoting girls' enrollment in education, affirmative action measures, and expanding technical and vocational training opportunities.
	CSOs recommendations on SRHR included in Policy and budget proposals in Kenya indicating support from relevant stakeholders	Key proposals from coalitions such as the SRHR Alliance and partners like GEM Trust include calls for removal of legal and policy barriers to adolescent access to sexual and reproductive health services and the enactment of a comprehensive Sexual and Reproductive Health Act to harmonise protections across law and policy. CSOs have also pushed for increased budget allocations for family planning commodities and reproductive health supplies, resulting in budget lines for procurement of family planning commodities (e.g., KSh 1.0B in the 2024/25 MTEF) and integrated reproductive health programmes supported through GoK/UNFPA partnerships.

		CSOs recommendations have influenced the inclusion of maternal and newborn unit upgrades and strengthened financing for youth-friendly SRH services in national and county planning frameworks, demonstrating a shift toward more rights-based SRHR funding in government budgets
	In Uganda and Tanzania SRHR IEC advocacy materials developed to enhance engagement between CSOs and duty bearers on inclusion of SRHR recommendations.	The findings could not clearly substantiate if Uganda and Tanzania have developed standardized SRHR IEC advocacy materials to enhance engagements between CSOs and duty bearers. However, discussions with CSOs from the two countries revealed that at organizational and coalition levels, they have developed IEC materials including toolkits and manuals on SRHR which are utilized in capacity strengthening and policy influencing spaces.
	Level of implementation of SRHR 4th cycle recommendations established in Kenya, resulting in review of the implementation plan.	The findings established that in Kenya, the State accepted and committed to implementing SRHR recommendations at its 4th cycle UPR, with public commitments to expand family planning and strengthen maternal health services, improve access to emergency obstetric care and enhance public awareness of sexual and reproductive health, including youth-specific services at the Human Rights Council adoption session. However, the level of implementation at baseline could not be clearly substantiated.
	UPR SRHR Implementation Plan developed in Uganda and Tanzania to align them with responsible government	The desk review findings confirmed that there is no publicly available Uganda-specific SRHR-only UPR implementation plan that focuses solely

	stakeholders, timelines, and monitoring indicators.	on sexual and reproductive health and rights (SRHR). The findings further noted that what <i>does</i> exist are general national UPR implementation and follow-up processes that civil society participates in. Similarly, the findings noted that Tanzania has not published a standalone UPR SRHR implementation plan that specifically organizes SRHR-related recommendations into an operational plan. The government participates in UPR cycles and accepts recommendations on a range of human rights issues, and civil society has conducted mid-term assessments on UPR implementation, noting progress and gaps across accepted recommendations.
	SRHR UPR recommendations reflected in government planning or budgetary session, with discussion on potential resource allocation or costing of recommended actions.	The findings highlighted that in Kenya, the government integrated UPR SRHR priorities such as expanding adolescent health services and improving maternal health into the Health Sector Medium-Term Expenditure Framework (MTEF), with budgeted lines for family planning commodities and Linda Mama maternity care scale-up that align with calls to remove barriers to reproductive health access. In Uganda, UPR emphasis on family planning and adolescent SRHR is mirrored in national budgeting for the Reproductive, Maternal, Newborn, Child and Adolescent Health (RMNCAH) programme, which allocates funding for contraceptive supplies and youth-friendly

		services as part of the Ministry of Health’s annual plans. In Tanzania, SRHR recommendations emphasizing youth access and service quality are reflected in the Health Sector Strategic Plan V and associated budgets that support youth-friendly reproductive health service standards and financing for reproductive health commodities under the One Plan III framework
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Table 1: Baseline Indicator Matrix

REFERENCES

United Nations High Commissioner for Refugees (UNHCR). (2021). *Minimum Initial Service Package and Sexual and Reproductive Health in Humanitarian Settings: Guidance Note*. Geneva: UNHCR.

EACHRights – About Us. Retrieved from: <https://www.eachrights.or.ke>

UPR Kenya – Stakeholder Submissions. Retrieved from: <https://www.upr-info.org/en/review/Kenya>

RAP4SRHR Program Brief – Access to Medicines Platform. Retrieved from: <https://accessmedsplatformkenya.org/wp-content/uploads/2024/03/RAPs-brief-final-1.pdf>

UPR Kenya – SRHR Thematic Group Submission. Retrieved from: <https://www.eachrights.or.ke/ssd/2024/10/SRHR-Thematic-Group-Report.pdf>

Dream Achievers Youth Organization – SRHR and Disability Toolkit. Retrieved from: <https://dreamachieverskenya.org/reports/DAYo-SRHR-BOOK-DESIGN.pdf>

FAWE Africa – Regional Advocacy and Policy Engagements. Retrieved from: <https://fawe.org/advocacy>

GEM Trust – About Us. Retrieved from: <https://gem.or.ke>

Feminist Republik – Partner Campaigns. Retrieved from: <https://feministrepublik.org/partners>

TICAH – Young Women’s SRHR Advocacy. Retrieved from: <https://ticah.org/programs/srhr>

FEMNET – Regional Feminist Advocacy Convenings. Retrieved from: <https://femnet.org>

Feminist Republik – Cross-Border Campaigns and Storytelling. Retrieved from : <https://feministrepublik.org>

CEHURD – Strategic Litigation Cases. Retrieved from: <https://www.cehurd.org/strategic-litigation>

CEHURD – Community Health Advocacy Reports. Retrieved from: <https://www.cehurd.org/publications>

NUDIPU – Disability and SRHR in Uganda. Retrieved from: <https://nudipu.org/resources>

SRHR Africa Trust – Regional Advocacy and Coalition Building. Retrieved from: <https://srhrafricatrust.org>

CWHRDs Tanzania – Annual Report 2023. Retrieved from: <https://www.cwhrds.or.tz/documents/2023%20WHRDsTZ-ANNUAL%20REPORT%20.pdf>

WGNRR Africa – Youth SRHR Training Toolkit. Retrieved from: <https://wgnrrafrica.org/sexual-and-reproductive-health-and-rights-srhr-training-toolkit>

UNFPA Tanzania – SRHR Outcomes for Young People. Retrieved from: <https://tanzania.unfpa.org/en/news/sexual-and-reproductive-health-and-rights-srhr-outcomes-young-people-still-alarming-state-many>

EAWHRDN – Regional Protection Mechanisms. Retrieved from: <https://eawhrdn.org>

African Union. (2003). Protocol to the African Charter on Human and Peoples’ Rights on the Rights of Women in Africa (Maputo Protocol). <https://au.int/en/treaties/protocol-african-charter-human-and-peoples-rights-rights-women-africa>

African Union. (2016). Maputo Plan of Action 2016–2030. (From: /mnt/data/32895-file-maputo_plan_of_action_english.pdf)

African Union. (2020). AU Continental Strategy on Sexual and Reproductive Health and Rights. (From: /mnt/data/30921-doc-srhr_english_0- AU Framework.pdf)

African Union. (2021). AU Gender Strategy 2022–2027. (From: /mnt/data/gender_equality_strategy_2022_-_2027- Tanzania.pdf)

AHRLJ. (2021). Koske, J. – SRHR in Africa. (From: /mnt/data/AHRLJ 1_2021 Koske.pdf)

Beijing Declaration and Platform for Action. (1995). (From: /mnt/data/BDPfA E.pdf)

Beijing+25 Report. (2021). Beijing25 Report. (From: /mnt/data/Beijing25_report_20210423_0.pdf)

CIVICUS. (2023). State of Civil Society Report. (Used externally; appears in your report — no file uploaded)

CHRAGG. (n.d.). Rights of LGBT Persons in Tanzania. (From: /mnt/data/rights-of-lgbt-persons-tanzania.pdf)

CSO UPR Stakeholder Report – Tanzania. (2021). (From: /mnt/data/CSOs UPR REPORT - 2021.pdf)

Dayo Kenya. (2021). SRHR & Disability Toolkit. (From: /mnt/data/Dream Achievers – DAYO link in report, not uploaded)

EACHRights. (2024). SRHR Thematic Group Report. (From: /mnt/data/SRHR-Thematic-Group-Report.pdf)

EAC. (2021). East African Community Sexual and Reproductive Health Bill 2021. (From: /mnt/data/Gazetted_-EAC_Sexual_and_Reproductive_Health_Bill_2021.pdf)

EAC. (2022). RMNCAH Policy Guidelines 2016–2030. (From: /mnt/data/EAC-RMNCAH-Policy-Guidelines-16-30.pdf)

FAWE. (2021). Policy and Advocacy Reports. (Referenced externally — not uploaded)

FEMNET. (2022). Kenya Factsheet 2022. (From: /mnt/data/Kenya-Factsheet_2022-FEMNET.pdf)

FEMNET. (2019). SOAW Report – Chapter 3: SRHR in AU Framework. (From: /mnt/data/SOAW-Report-Chapter-3-SRHR-in-the-AU-Framework.pdf)

FEMNET. (2020). SOAW Popular Version. (From: /mnt/data/SOAW-Report-POPULAR-VERSION.pdf)

FGM Act. (2011). Prohibition of Female Genital Mutilation Act No. 32 of 2011. (From: /mnt/data/ProhibitionofFemaleGenitalMutilationAct_No32of2011.pdf)

Government of Kenya. (2006). HIV & AIDS Prevention and Control Act No.14 of 2006. (From: /mnt/data/HIVandAIDSPreventionandControlAct_No14of2006.pdf)

Government of Kenya. (2006). Sexual Offences Act No. 3 of 2006. (From: /mnt/data/SexualOffencesAct_No3of2006.pdf)

Government of Kenya. (2010). The Constitution of Kenya 2010. (From: /mnt/data/The_Constitution_of_Kenya_2010.pdf)

Government of Kenya. (2022). National Reproductive Health Policy 2022–2032. (From: /mnt/data/The_National_Reproductive_Health_Policy_2022_-_2032- Kenya.pdf)

Government of Kenya. (2023). Social Health Authority Act (SHA). (Referenced; document not uploaded)

Government of Kenya. (n.d.). National Guidelines for Youth-Friendly Services. (From: /mnt/data/National_guidelines_for_provision_of_youth_friendly_services.pdf)

Government of Kenya. (2019). National Gender and Development Policy. (From: /mnt/data/NATIONAL-POLICY-ON-GENDER-AND-DEVELOPMENT.pdf)

Health Rights Advocacy Report – KNCHR. (2021). (From: /mnt/data/Health-Rights-Review-Report Kenya.pdf)

Health Rights Review Report – Regional. (2021). (From: /mnt/data/Health-Rights-Review-Report.pdf)

HOYMAS. (2024). Annual Report. (From: /mnt/data/Annual Report (2024 - 2025) amended_1.pdf)

IOM & AU. (2023). ESAR SRHR Framework. (From: /mnt/data/iom-esa-srhr-framework-web-version-191023.pdf)

IPPF. (2022). UMATI Annual Report. (From: /mnt/data/kco_annual_report_final_report.pdf)

Kenya Human Rights Commission. (2021). Annual Report. (From: /mnt/data/KHRC-April-2020-March-2021-Annual-Report.pdf)

KELIN Kenya. (2024). UPR Kenya SRHR Thematic Group Report. (From: /mnt/data/UPR-Kenya_SRHR-Thematic-Group-report.pdf)

KELIN Kenya. (2024). UPR Kenya SRHR Thematic Group Report (duplicate). (From: /mnt/data/UPR-Kenya_SRHR-Thematic-Group-report (1).pdf)

KNCHR. (2012). Reproductive Health Rights Report. (From: /mnt/data/reproductive_health_report KNHRC.pdf)

Plan International Kenya. (2019). SRHR Training Manual. (From: /mnt/data/srhr_training_manual_1st_oct_2019_plan_kenya.pdf)

Policy Brief on Adolescent SRHR – RHRN. (2022). (From: /mnt/data/Policy-Recommendations-For-The-National-Adolescent-Sexual-And-Reproductive-Health-Policy-A-Policy-Brief-By-Rhrn-Kenya-Platform-.pdf)

Policy Brief on Poverty & SRHR in Kenya. (2021). (From: /mnt/data/Policy-Brief-30-Addressing-Sexual-Reproductive-Health-Challenges-in-Kenya-The-Poverty-Dimension.pdf)

Penal Code of Kenya. (2009). (From: /mnt/data/PenalCodeCap63.pdf)

SADC. (2019). SADC SRHR Strategy 2019–2030. (From: /mnt/data/Final_SADC_SRHR Strategy.pdf)

SHRH UHC Report. (n.d.). SRHR as an Essential Element of UHC. (From: /mnt/data/SRHR an essential element of UHC.pdf)

SRHR & Climate Policy Report. (2023). (From: /mnt/data/Sexual and Reproductive Health and Rights (SRHR) in climate polic.pdf)

SRHR Assessment Baseline Report. (2025). (From: /mnt/data/SRHR-Assessment-Report-Baseline-Report_FINAL_2025-01-22-121404_zhok.pdf)

Tanzania National Adolescent Health & Development Strategy. (2018). (From: /mnt/data/Tanzania-National-Adolescent-Health-and-Development-Strategy.pdf)

Tanzania Constitution. (1985). (From: /mnt/data/constitution_of_the_united_republic_of_tanzania_last_amended_1985_ref world.pdf)

Tanzania Media Services Act. (2016). (From: /mnt/data/sw-1751354186-The Media Services Act.pdf)

Tanzania NGO Act. (2019). (From: /mnt/data/sw-1751355963-THE NON-GOVERNMENTAL ORGANISATIONS ACT.pdf)

Tanzania Sexual Offences Special Provisions Act. (1998). (From: /mnt/data/tanzania_sexualoffences_1998_en.pdf)

Tanzania UPR Submission. (2021). (From: /mnt/data/Tanzania_UPR-Submsn_-Mar-2021-website.pdf)

Tanzania UHI Act – Policy Brief. (2023). (From: /mnt/data/Tanzania-UHI-Act-2023-Policy-brief.pdf)

TGNP. (2021). Strategic Plan 2021–2025. (From: /mnt/data/TGNP-STRATEGIC-PLAN-2021-2025.pdf)

TGNP. (2018). Gender Assessment. (In report; file corresponds to /mnt/data/Gender-Policy-and-Guidelines_2023 Tanzania.pdf)

UNHCR. (2021). Sexual and Reproductive Health in Humanitarian Settings: Guidance Note. (From: /mnt/data/Annex C SRH- UNHCR.pdf)

UNHCR. (2023). Minimum Initial Service Package for SRHR. (From: /mnt/data/UNHCR-Minimum Inital Service Package for....pdf)

UNFPA. (2021). Regional Legal Framework on ASRHR. (From: /mnt/data/final_-_regional_legal_framework_on_asrhr_1- UNFPA.pdf)

WHO. (2022). Global Strategy for Women's, Children's and Adolescents' Health. (From: /mnt/data/A76_5-en WHO Global Strategy.pdf)

World Bank. (2023). UHC in Tanzania – Missing Links in Preventive SRHR. (From: /mnt/data/The missing link in Kenya s universal health coverage experiment a preventive and promotive approach to SRHR.pdf)

Amplify Change. *Our Work*. <https://amplifychange.org/our-work>

CSA Kenya. *UPR Kenya SRHR Thematic Group Report (2024)*. <https://csakenya.org>

SRHR Alliance Uganda. *Youth Demand Action*. <https://srhrallianceuganda.org>

Restless Development. *Policy Analysis of Adolescent SRHR in Tanzania (2025)*. <https://restlessdevelopment.org>

EACHRights. *Official Website*. <https://eachrights.or.ke>

UGANET (2022): Issue Paper Analysing Laws and Policies Affecting SRHR in Uganda

Alexander Kagaha, Lenore Manderson: *Power, Policy And Abortion Care In Uganda (2020)*